

Tue, 25 February 2025

STATE OF TEXAS §

This is to certify that I, Jim Markham, am employed by the Texas Department of Transportation (Department); that I am the Custodian of Motor Vehicle Crash Records for such Department; that the attached is a true and correct copy of the peace officer's report filed with the Department referred to in the attached request with the crash date of Fri, 21 February 2025, which occurred in Dallas County; that the investigations of motor vehicle crashes by peace officers are authorized by law; that this Texas Peace Officer's Crash Report is required by law to be completed and filed with this Department; that this report sets forth matters observed pursuant to duty imposed by law as to which matters there was a duty to report, or factual findings resulting from an investigation made pursuant to authority granted by law.



Jim Markham
Director, Crash Data & Analysis Section
Traffic Safety Division
125 East 11th Street
Austin, TX 78701-2483



☐ FATAL ☐ CMV ☐ SCHOOL BUS ☐ RAILROAD ☐ MAB ☐ SUPPLEMENT ☐ ZONE

Texas Peace Officer's Crash Report (Form CR-3 4/1/2023)

Refer to the attached code sheet for numbered fields

Questions? Call 844/274-7457

*=These fields are required on all additional sheets submitted for this crash (ex : additional vehicles, occupants, injured, etc.)

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*Crash Date (MM/DD/YYYY)	0 / 2 / 21 / 2025	*Crash Time (24HRMM)	1 8 0 0	Case ID	25-011935	Local Use	TD01													
*County Name	DALLAS	*City Name	DALLAS					☐ Outside City Limit												
In your opinion, did this crash result in at least \$1000 damage to any one person's property?	☒ Yes ☐ No	Latitude (decimal degrees)					Longitude — (decimal degrees)													
ROAD ON WHICH CRASH OCCURRED																				
*1 Rdwy. Sys.	IH	*Hwy. Num.	35E	2 Rdwy. Part	1	Block Num.	10900	3 Street Prefix	N	*Street Name	STEMMONS	4 Street Suffix	FWY							
☐ Private Drive or Road, Private Property, Parking Lot	3 Dir. of Traffic	N	☐ Toll Road/ Toll Lane	Speed Limit	65	Const. Zone	☐ Yes ☒ No	Workers Present	☐ Yes ☒ No	Secondary Crash	☐ Yes ☒ No	Street Desc.								
INTERSECTING ROAD, OR IF CRASH NOT AT INTERSECTION, NEAREST INTERSECTING ROAD OR REFERENCE MARKER																				
At Int.	☐ Yes ☒ No	1 Rdwy. Sys.	LR	Hwy. Num.		2 Rdwy. Part	1	Block Num.		3 Street Prefix		Street Name	WALNUT HILL	4 Street Suffix	AVE					
Distance from Int. or Ref. Marker	1000	☒ FT ☐ MI	3 Dir. from Int. or Ref. Marker	S	Ref. Marker		Speed Limit		Street Desc.		RRX Num.									
Unit Num.	1	5 Unit Desc.	1	☐ Parked Vehicle	☐ Hit and Run	LP State	TX	LP Num.	VDX1349	VIN	3 N 1 C N 7 A P 0 H L 8 5 1 6 2 4									
Veh. Year	2 0 1 7	6 Veh. Color	WHI	Veh. Make	NISSAN	Veh. Model	VERSA	7 Body Style	P4											
☐ Responder Struck (Explain in Narrative if checked)				8 Autonomous Unit	NO	9 Autonomous Level Engaged	NO AUTOMATION			☐ Police, Fire, EMS on Emergency (Explain in Narrative if checked)										
10 DL/ID Type	1	DL/ID State	TX	DL/ID Num.	50253917	11 DL Class	C	12 CDL End.	96	13 DL Rest.	96	DOB (MM/DD/YYYY)	0 2 / 2 5 / 2 0 0 4							
Address (Street, City, State, ZIP) 2824 LAKE GARDENS DR IRVING 75060																				
Person Num.	14 Prsn. Type	15 Seat Position	Name: Last, First, Middle Enter Driver or Primary Person for this Unit on first line				16 Injury Severity	Age	17 Ethnicity	18 Sex	19 Eject.	20 Restr.	21 Airbag	22 Helmet	23 Sol.	24 Alc. Spec.	Alc. Result	25 Drug Spec.	26 Drug Result	27 Drug Category
1	1	1	VERON, KYARA				N	20	H	2	1	1	2	97	N	96		96	97	97
														Not Applicable - Alcohol and Drug Results are only reported for Driver/Primary Person for each Unit.						
☒ Owner ☐ Lessee	Owner/Lessee Name & Address VERON, KYARA, 2824 LAKE GARDENS DR IRVING 75060																			
Proof of Fin. Resp.	☒ Yes ☐ No	Expired Exempt	28 Fin. Resp. Type	2	Fin. Resp. Name	FOREMOST COUNTY MUTUAL INS. CO.				Fin. Resp. Num.	43G01499812100									
Fin. Resp. Phone Num.	(800) 527-3907				29 Vehicle Damage Rating 1	F D 3				29 Vehicle Damage Rating 2	B D 2				Vehicle Inventoried	☐ Yes ☒ No				
Towed By					Towed To															
Unit Num.	2	5 Unit Desc.	1	☐ Parked Vehicle	☐ Hit and Run	LP State	TX	LP Num.	TVR4944	VIN	5 F N R L 5 H 4 5 F B 0 3 3 2 4 8									
Veh. Year	2 0 1 5	6 Veh. Color	WHI	Veh. Make	HONDA	Veh. Model	ODYSSEY	7 Body Style	VN											
☐ Responder Struck (Explain in Narrative if checked)				8 Autonomous Unit	NO	9 Autonomous Level Engaged	NO AUTOMATION			☐ Police, Fire, EMS on Emergency (Explain in Narrative if checked)										
10 DL/ID Type	1	DL/ID State	TX	DL/ID Num.	50292857	11 DL Class	C	12 CDL End.	96	13 DL Rest.	96	DOB (MM/DD/YYYY)	0 5 / 3 0 / 1 9 8 4							
Address (Street, City, State, ZIP) 6936 PADDINGTON LN FRISCO, TX 75034																				
Person Num.	14 Prsn. Type	15 Seat Position	Name: Last, First, Middle Enter Driver or Primary Person for this Unit on first line				16 Injury Severity	Age	17 Ethnicity	18 Sex	19 Eject.	20 Restr.	21 Airbag	22 Helmet	23 Sol.	24 Alc. Spec.	Alc. Result	25 Drug Spec.	26 Drug Result	27 Drug Category
1	1	1	BUDATI, VENKATA, RAMANJANEYULU				N	40	A	1	1	1	2	97	N	96		96	97	97
														Not Applicable - Alcohol and Drug Results are only reported for Driver/Primary Person for each Unit.						
☒ Owner ☐ Lessee	Owner/Lessee Name & Address XPRESS FOODS LLC, 6936 PADDINGTON LN FRISCO, TX 75034																			
Proof of Fin. Resp.	☒ Yes ☐ No	Expired Exempt	28 Fin. Resp. Type	2	Fin. Resp. Name	PROGRESSIVE (COUNTY MUTUAL) INS. CO.				Fin. Resp. Num.	975164607									
Fin. Resp. Phone Num.	(800) 776-4737				29 Vehicle Damage Rating 1	F D 2				29 Vehicle Damage Rating 2					Vehicle Inventoried	☐ Yes ☒ No				
Towed By					Towed To															

DISPOSITION OF INJURED/KILLED	Unit Num.	Prsn. Num.	Taken To	Taken By	Date of Death (MM/DD/YYYY)	Time of Death (24HRMM)

CHARGES	Unit Num.	Prsn. Num.	Charge	Citation/Reference Num.

DAMAGE	Damaged Property Other Than Vehicles	Owner's Name	Owner's Address

CMV	Unit Num.	☐ 10,001+ LBS.	☐ Transporting Hazardous Material	☐ 9+ Capacity	CMV Disabling Damage?	☐ Yes ☐ No	30 Veh. Oper.	31 Carrier ID Type	Carrier ID Num.	
	Carrier's Corp. Name				Carrier's Primary Addr.				32 Veh. Type	
	33 Bus Type	☐ RGWV ☐ GVWR	HazMat Released	☐ Yes ☐ No	34 HazMat Class Num.	HazMat ID Num.	34 HazMat Class Num.	HazMat ID Num.	35 Cargo Body Type	
	Unit Num.	☐ RGWV ☐ GVWR	36 Trlr. Type	CMV Disabling Damage?	☐ Yes ☐ No	Unit Num.	☐ RGWV ☐ GVWR	36 Trlr. Type	CMV Disabling Damage?	☐ Yes ☐ No
	Sequence Of Events	37 Seq. 1	37 Seq. 2	37 Seq. 3	37 Seq. 4	Intermodal Shipping Container Permit	☐ Yes ☐ No	Actual Gross Weight	Total Num. Axles	

FACTORS & CONDITIONS	38 Contributing Factors (Investigator's Opinion)					39 Vehicle Defects (Investigator's Opinion)					Environmental and Roadway Conditions						
	Unit #	Contributing		May Have Contrib.		Contributing		May Have Contrib.		40 Weather Cond.	41 Light Cond.	42 Entering Roads	43 Roadway Type	44 Roadway Alignment	45 Surface Condition	46 Traffic Control	
	1	44															
	2	44								1	3	97	3	2	1	17	

NARRATIVE AND DIAGRAM	Investigator's Narrative Opinion of What Happened (Attach Additional Sheets if Necessary)	Indicate North	Field Diagram - Not to Scale
	UNITS 1, 2 AND 3 WERE TRAVELING NORTH BOUND ON IH 35 IN THE LEFT CENTER LANE. UNIT 1 DRIVER STATED SHE LOOKED DOWN ON HER PHONE AND COULD NOT STOP ON TIME AND STRUCK UNIT 3. UNIT 1 DRIVER STATED THAT UNIT 3 DROVE AWAY FROM THE SCENE OF THE ACCIDENT. UNIT 2 DRIVER STATED THAT UNIT 1 MADE A SUDDEN STOP AND STRUCK UNIT 1. UNIT 3 DRIVER LEFT THE SCENE OF THE ACCIDENT WITHOUT PROVIDING THE REQUIRED DOCUMENTS.		

INVESTIGATOR	Date Notified (MM/DD/YYYY)	02 / 21 / 2025	Time Notified (24HRMM)	1 8 3 5	How Notified	RADIO DISPATCHED		
	Date Arrived (MM/DD/YYYY)	02 / 21 / 2025	Time Arrived (24HRMM)	1 8 4 9	Report Date (MM/DD/YYYY)	02 / 21 / 2025		
	Date Roadway Cleared (MM/DD/YYYY)	02 / 21 / 2025	Time Roadway Cleared (24HRMM)	1 8 4 9	Date Scene Cleared (MM/DD/YYYY)	02 / 21 / 2025	Time Scene Cleared (24HRMM)	1 9 1 3
	Investigation Complete	☒ Yes ☐ No	Investigator Name (Printed)	Medina, Jesus			ID Num.	1248
	ORI Num.	T X 0 5 7 0 0 0 0	*Agency	DALLAS COUNTY SHERIFF'S OFFICE			Service/Region/DA	T S 3

3	Num. Prsns
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Questions? Call 844/274-7457

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*Crash Date (MM/DD/YYYY)		0	2	/	2	1	/	2	0	2	5	*Crash Time (24HRMM)		1		8		0		0	Case ID	25-011935						Local Use	TD01																																					
*County Name DALLAS												*City Name DALLAS																									☐ Outside City Limit																													
In your opinion, did this crash result in at least \$1000 damage to any one person's property?												☒	Yes												Latitude (decimal degrees)												Longitude — (decimal degrees)																													
No																																																																		
ROAD ON WHICH CRASH OCCURRED																																																																		
*1 Rdwy. Sys.			IH	*Hwy. Num.			35E	2 Rdwy. Part			1	Block Num.			10900	3 Street Prefix			N	*Street Name			STEMMONS	4 Street Suffix			FWDY																																							
☐ Private Drive or Road, Private Property, Parking Lot			3 Dir. of Traffic			N	☐ Toll Road/ Toll Lane			Speed Limit			65	Const. Zone			☐ Yes ☒ No	Workers Present			☐ Yes ☒ No	Secondary Crash			☐ Yes ☒ No	Street Desc.																																								
INTERSECTING ROAD, OR IF CRASH NOT AT INTERSECTION, NEAREST INTERSECTING ROAD OR REFERENCE MARKER																																																																		
At Int.			☐ Yes ☒ No	1 Rdwy. Sys.			LK	Hwy. Num.				2 Rdwy. Part			1	Block Num.				3 Street Prefix				Street Name			WALNUT HILL	4 Street Suffix			AWE																																			
Distance from Int. or Ref. Marker			1000	☒ FT ☐ MI			3 Dir. from Int. or Ref. Marker			S	Ref. Marker				Speed Limit				Street Desc.				RRX Num.																																											
Unit Num.			3	5 Unit Desc.			1	☐ Parked Vehicle			☒ Hit and Run			LP State			UN	LP Num.				VIN																																												
Veh. Year				6 Veh. Color			99	Veh. Make			UNKNOWN						Veh. Model			UNKNOWN						7 Body Style			99																																					
☐ Responder Struck (Explain in Narrative if checked)													8 Autonomous Unit UNKNOWN										9 Autonomous Level Engaged UNKNOWN										☐ Police, Fire, EMS on Emergency (Explain in Narrative if checked)																																	
10 DL/ID Type			99			DL/ID State			UN			DL/ID Num.						11 DL Class			99			12 CDL End.			99			13 DL Rest.			99			DOB (MM/DD/YYYY)																														
Address (Street, City, State, ZIP)																																																																		
Person Num.			Name: Last, First, Middle			Enter Driver or Primary Person for this Unit on first line										16 Injury Severity			Age			17 Ethnicity			18 Sex			19 Eject.			20 Restr.			21 Airbag			22 Helmet			23 Sol.			24 Alc. Spec.			Alc. Result			25 Drug Spec.			26 Drug Result			27 Drug Category											
1			1			UNKNOWN, UNKNOWN										99						99			99			99			99			99			97			N			96						96			97			97											
☒ Owner ☐ Lessee			Owner/Lessee Name & Address UNKNOWN, UNKNOWN																																																															
Proof of Fin. Resp.			☐ Yes ☒ No			☐ Expired ☐ Exempt			28 Fin. Resp. Type			Fin. Resp. Name			Fin. Resp. Num.																																																			
Fin. Resp. Phone Num.						29 Vehicle Damage Rating 1						29 Vehicle Damage Rating 2						Vehicle Inventoried			☐ Yes ☒ No																																													
Towed By						Towed To																																																												
Unit Num.			5 Unit Desc.			☐ Parked Vehicle			☐ Hit and Run			LP State			LP Num.			VIN																																																
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Fin. Resp. Phone Num.						29 Vehicle Damage Rating 1						29 Vehicle Damage Rating 2						Vehicle Inventoried			☐ Yes ☐ No																																													
Towed By						Towed To																																																												

Law Enforcement and TxDOT Use ONLY. Form CR-3 (Rev. 4/1/2023)		Case ID 25-011935	TxDOT Crash ID 20667596.1/2025071260		Page 4 of 4															
DISPOSITION OF INJURED/KILLED	Unit Num.	Prsn. Num.	Taken To		Taken By		Date of Death (MM/DD/YYYY)		Time of Death (24HRMM)											
CHARGES	Unit Num.	Prsn. Num.	Charge						Citation/Reference Num.											
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	Carrier's Corp. Name				Carrier's Primary Addr.				32 Veh. Type											
	33 Bus Type	☐ RGWW ☐ GVWR	HazMat Released	☐ Yes ☐ No	34 HazMat Class Num.	HazMat ID Num.		34 HazMat Class Num.	HazMat ID Num.	35 Cargo Body Type										
	Unit Num.	☐ RGWW ☐ GVWR	36 Trlr. Type		CMV Disabling Damage?	☐ Yes ☐ No	Unit Num.	☐ RGWW ☐ GVWR	36 Trlr. Type	CMV Disabling Damage?	☐ Yes ☐ No									
	Sequence Of Events	37 Seq. 1	37 Seq. 2	37 Seq. 3	37 Seq. 4	Intermodal Shipping Container Permit	☐ Yes ☐ No	Actual Gross Weight		Total Num. Axles										
FACTORS & CONDITIONS	38 Contributing Factors (Investigator's Opinion)					39 Vehicle Defects (Investigator's Opinion)					Environmental and Roadway Conditions									
	Unit #	Contributing		May Have Contrib.		Contributing		May Have Contrib.		40 Weather Cond.	41 Light Cond.	42 Entering Roads	43 Roadway Type	44 Roadway Alignment	45 Surface Condition	46 Traffic Control				
NARRATIVE AND DIAGRAM	Investigator's Narrative Opinion of What Happened (Attach Additional Sheets if Necessary)										Field Diagram - Not to Scale									
INVESTIGATOR	Date Notified (MM/DD/YYYY)					Time Notified (24HRMM)					How Notified									
	0 2 / 2 1 / 2 0 2 5					1 8 3 5					RADIO DISPATCHED									
	Date Arrived (MM/DD/YYYY)					Time Arrived (24HRMM)					Report Date (MM/DD/YYYY)									
	0 2 / 2 1 / 2 0 2 5					1 8 4 9					0 2 / 2 1 / 2 0 2 5									
	Date Roadway Cleared (MM/DD/YYYY)					Time Roadway Cleared (24HRMM)					Date Scene Cleared (MM/DD/YYYY)					Time Scene Cleared (24HRMM)				
0 2 / 2 1 / 2 0 2 5					1 8 4 9					0 2 / 2 1 / 2 0 2 5					1 9 1 3					
Investigation Complete		☒ Yes ☐ No		Investigator Name (Printed)										ID Num.						
				Medina, Jesus										1248						
ORI Num.		T X 0 5 7 0 0 0 0		*Agency										Service/Region/DA						
				DALLAS COUNTY SHERIFF'S OFFICE										T S 3						



Q Factor Chiropractic
100 E. Royal Lane Suite 125
Irving, TX 75039

Statement ID: 311779-126260617-22180750		
Statement Date	Pay This Amount	Chart ID
04/04/2025	\$10,826.01	VEKY000001
SHOW AMOUNT PAID HERE:	\$	

Kyara Veron
2824 Lake gardens dr
Irving, TX 75060

MAKE CHECKS PAYABLE / REMIT TO:

Q Factor Chiropractic
100 E. Royal Lane Suite 125
Irving, TX 75039

For questions about billing, call (214) 920-9111.

.....
Please detach and return top portion with your payment.

Patient: Kyara Veron	Chart ID: VEKY000001	Statement ID: 311779-126260617-22180750
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Date of Service	Provider	Procedure	Mods	Charge	Adjmt	Ins. Paid	Pt Paid	Bal Due
2/24/2025 02:00PM	Dr. Edward Amadi, DC	72050 - X-RAY EXAM NECK SPINE 4/5VWS	:::	\$345.00				\$345.00
2/24/2025 02:00PM	Dr. Edward Amadi, DC	72070 - X-RAY EXAM THORAC SPINE 2VWS	:::	\$212.50				\$212.50
2/24/2025 02:00PM	Dr. Edward Amadi, DC	72100 - X-RAY EXAM L-S SPINE 2/3 VWS	:::	\$216.25				\$216.25
2/24/2025 02:00PM	Dr. Edward Amadi, DC	99455 - WORK RELATED DISABILITY EXAM	:::	\$500.00				\$500.00
2/24/2025 02:00PM	Dr. Edward Amadi, DC	73560 - X-RAY EXAM OF KNEE 1 OR 2	:::	\$167.50				\$167.50
Total:				\$1,441.25				\$1,441.25

2/24/2025 02:00PM	Dr. Edward Amadi, DC	99212 - OFFICE O/P EST SF 10 MIN	25:::	\$129.65				\$129.65
2/24/2025 02:00PM	Dr. Edward Amadi, DC	99203 - OFFICE O/P NEW LOW 30 MIN	:::	\$301.20				\$301.20
2/24/2025 02:00PM	Dr. Edward Amadi, DC	98941 - CHIROPRACT MANJ 3-4 REGIONS	:::	\$109.75				\$109.75
2/24/2025 02:00PM	Dr. Edward Amadi, DC	A9273 - Hot/cold bottle/cap/col/wrap	:::	\$56.25				\$56.25
2/24/2025 02:00PM	Dr. Edward Amadi, DC	E0730 - Tens four lead	NU:::	\$550.00				\$550.00
2/24/2025 02:00PM	Dr. Edward Amadi, DC	97014 - ELECTRIC STIMULATION THERAPY	:::	\$43.75				\$43.75

Date of Service	Provider	Procedure	Mods	Charge	Adjmt	Ins. Paid	Pt Paid	Bal Due
2/24/2025 02:00PM	Dr. Edward Amadi, DC	97010 - HOT OR COLD PACKS THERAPY	:::	\$31.25				\$31.25
				Total:	\$1,221.85			\$1,221.85
2/25/2025 11:00AM	Dr. Edward Amadi, DC	97014 - ELECTRIC STIMULATION THERAPY	:::	\$43.75				\$43.75
2/25/2025 11:00AM	Dr. Edward Amadi, DC	97010 - HOT OR COLD PACKS THERAPY	:::	\$31.25				\$31.25
				Total:	\$75.00			\$75.00
2/26/2025 11:00AM	Dr. Edward Amadi, DC	97014 - ELECTRIC STIMULATION THERAPY	:::	\$43.75				\$43.75
2/26/2025 11:00AM	Dr. Edward Amadi, DC	97010 - HOT OR COLD PACKS THERAPY	:::	\$31.25				\$31.25
				Total:	\$75.00			\$75.00
2/27/2025 09:45AM	Dr. Edward Amadi, DC	97014 - ELECTRIC STIMULATION THERAPY	:::	\$43.75				\$43.75
2/27/2025 09:45AM	Dr. Edward Amadi, DC	97010 - HOT OR COLD PACKS THERAPY	:::	\$31.25				\$31.25
				Total:	\$75.00			\$75.00
2/28/2025 06:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
				Total:	\$388.32			\$388.32
3/03/2025 11:00AM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
				Total:	\$388.32			\$388.32
3/04/2025 11:00AM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
				Total:	\$388.32			\$388.32
3/05/2025 04:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
				Total:	\$388.32			\$388.32
3/06/2025 03:30PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
				Total:	\$388.32			\$388.32

Date of Service	Provider	Procedure	Mods	Charge	Adjmt	Ins. Paid	Pt Paid	Bal Due
3/07/2025 03:30PM	Dr. Edward Amadi, DC	99213 - OFFICE O/P EST LOW 20 MIN	:::	\$206.30				\$206.30
3/07/2025 03:30PM	Dr. Edward Amadi, DC	98941 - CHIROPRACT MANJ 3-4 REGIONS	:::	\$109.75				\$109.75
Total:				\$316.05				\$316.05
3/10/2025 03:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/11/2025 03:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/12/2025 04:30PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/13/2025 11:00AM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/14/2025 11:00AM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/17/2025 04:30PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/18/2025 04:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/19/2025 12:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/20/2025 04:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32

Date of Service	Provider	Procedure	Mods	Charge	Adjmt	Ins. Paid	Pt Paid	Bal Due
Total:				\$388.32				\$388.32
3/21/2025 03:00PM	Dr. Edward Amadi, DC	99213 - OFFICE O/P EST LOW 20 MIN	:::	\$206.30				\$206.30
3/21/2025 03:00PM	Dr. Edward Amadi, DC	98941 - CHIROPRACT MANJ 3-4 REGIONS	:::	\$109.75				\$109.75
Total:				\$316.05				\$316.05
3/24/2025 04:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/25/2025 04:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/26/2025 04:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/27/2025 04:00PM	Dr. Edward Amadi, DC	97110 - THERAPEUTIC EXERCISES	:::	\$388.32				\$388.32
Total:				\$388.32				\$388.32
3/28/2025 03:30PM	Dr. Edward Amadi, DC	99213 - OFFICE O/P EST LOW 20 MIN	:::	\$206.30				\$206.30
3/28/2025 03:30PM	Dr. Edward Amadi, DC	98941 - CHIROPRACT MANJ 3-4 REGIONS	:::	\$109.75				\$109.75
Total:				\$316.05				\$316.05

Patient Payment Plans/Memberships

Due Date	Provider	Membership Name	Bal Due
			Total Amount: \$10,826.01

For questions about billing, call (214) 920-9111.

Q Factor Chiropractic, 100 E. Royal Lane Suite 125, Irving, TX 75039



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/24/2025 2:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Check-In Reasons for Visit:

Kyara noted the following complaints: cervical, thoracic, lumbar, Knee (right). Patient described pain as 6/10 on a scale of 0 to 10 with 0 being the least pain and 10 being the most pain. Patient complains they have felt an aching, "tightness" discomfort at the chief complaint areas listed in this note. Kyara states that since the date of the accident, the overall condition and complaints have worsened.

HISTORY:

- Frequency/Quality: Patient describes discomfort caused by accident as Frequent (<75% but >50% of the time)
 - Modifying factors: Relieved by: (nothing)
 - ADL/Functional Deficits: Explains the following activities/movement has become difficult due to their accident: getting up/sitting down, bending, turning
 - Patient reports status of the following condition(s) and diseases which may relate to complaint(s): Thyroid, Asthma
 - Patient has confirmed they are not currently pregnant.
 - Surgery: none
 - Medications: The patient is currently taking prescribed medication for: thyroid, (Other).
- The patient stated their smoking frequency to be the following: smoke occasionally.
The patient stated the frequency of their drinking as: (non-drinker).

Kyara Veron was seen today for complaint(s) resulting from a **Auto Accident (Personal)** incident having completed the patient intake questionnaire. The questionnaire was reviewed and annotated by the examining provider as needed. The completed questionnaire is in the patient's permanent digital file and available for review. She signed consent for evaluation and possible treatment of injuries sustained as the result of the accident that occurred on or about **02/21/2025**.

Kyara was positioned in the driver position of the vehicle, and when questioned about wearing seatbelts, she replied she was restrained. Kyara reports that she was looking straight ahead at the time of the impact and hit the following vehicle interior parts: airbag. Patient stated she did receive a head injury and did not lose consciousness. The patient's vehicle was impacted in the following location(s): rear end The patient's vehicle movement was moving forward. The damage assessment of the patient's vehicle was heavy visible damage. The patient's car was towed from the scene.

Police did arrive at the scene and an accident report was completed. EMS was not at the scene. Kyara arranged for a ride home from the scene. The patient was seen by the following providers: (none).

Daily Encounter:

Treatment for acute/active care on **Visit #1**
Office Visit



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/24/2025 2:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right). 5-6/10 reported using the Visual Analog Scale. The pain was described as achey, dull, soreness, stiffness.

She reports her ability to participate with walking/standing, sitting, moving head, bending over at the waist with an increase in pain.

Hypothyroidism

Asthma

patient get headaches off and on

Daily Encounter: treatment for acute/active care on 02/24/2025

INITIAL EXAMINATION:

Kyara was seen today for their initial examination of the condition currently being treated for the injuries sustained due to their accident. The findings are as follows:

Objective:

Musculoskeletal - Range of Motion - Knee - Active:

-Flexion: 130 /130 degrees w/o pain

-Hyperextension: 0/0 to 15 degrees w/o pain **Ortho-** Cervical Distraction Test performed. Patient indicated no pain.

Ortho- Shoulder Depression Test performed. Patient indicated pain bilaterally. **Ortho-** Maximal Compression Test

performed. Patient indicated pain bilaterally. Ortho-Straight Leg Raiser Test performed. Patient indicated pain

bilaterally. Ortho-Kemp's Test performed. Patient indicated pain bilaterally. **Ortho-**Knee Distraction Test performed.

Patient indicated local pain right side.

Musculoskeletal - Range of Motion - Cervical

-Flexion: 50/60 degrees

-Extension: 45/55 degrees

-Left Lat. Flexion: 30/40 degrees

-Right Lat. Flexion: 20/40 degrees

-Left Rotation: 70/80 degrees

-Right Rotation: 60/80 degrees



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/24/2025 2:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Musculoskeletal - Range of Motion - ThoracoLumbar

-Flexion: 85/90 degrees

-Extension: 25/30 degrees

-Left Lat. Flexion: 30/35 degrees

-Right Lat. Flexion: 20/35 degrees

-Left Rotation: 25/30 degrees

-Right Rotation: 15/30 degrees

Neuro-Sensory-Pain: Evaluations performed bilaterally. Dermatomal normal findings at all upper & lower spinal segments.

Neuro-Light Touch: Evaluations performed and indicated light touch sensations were within normal limits at all upper & lower spinal segments.

Neuro-Deep Tendon Reflexes (normal 2+):

- Biceps: Left 2+, Right 2+,

- Triceps: Left 2+, Right 2+,

- Brachioradialis: Left 2+, Right 2+,

- Patellar: Left 2+, Right 2+,

- Achilles: Left 2+, Right 2+,

Neuro-Upper extremity resistive isometric motor testing (normal 5/5):

- Shoulder Elevation: Left: 5 / 5 Right: 5 / 5.

- Deltoid: Left: 5 / 5 Right: 5 / 5

- Biceps: Left: 5 / 5 Right: 5 / 5

- Triceps: Left: 5 / 5 Right: 5 / 5

- Wrist Flexors: Left: 5 / 5 Right: 5 / 5

- Wrist Extensors: Left: 5 / 5 Right: 5 / 5

- Finger Extensors: Left: 5 / 5 Right: 5 / 5

- Finger Flexors: Left: 5 / 5 Right: 5 / 5



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/24/2025 2:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

-
- Finger Abductors: Left: 5/ 5 Right: 5 / 5
 - Palmar Interossei: Left: 5 / 5 Right: 5 / 5

Neuro-Lower extremity resistive isometric motor testing (normal 5/5):

- Iliopsoas: Left: 5 / 5 Right: 5 / 5
- Quadriceps: Left: 5 / 5 Right: 5 / 5
- Anterior Tibialis: Left: 5/ 5 Right: 5 / 5
- Hallucis Longus: Left: 5 / 5 Right: 5 / 5
- Ext Digitorum Longus & Brevis: Left: 5 / 5 Right: 5 / 5
- Gluteus Medius: Left: 5 / 5 Right: 5 / 5

Musculoskeletal

- Gait and Station: normal gait and normal balance
- Inspection/Percussion +/- Palpation: cervical erector paraspinal, thoracic and lumbar erector paraspinal, piriformis bilateral
- Postural Analysis: Short left leg (pelvic deficiency)
- Spinal Stability/Restriction(s)/Subluxation(s): C5, C7-T1, T5-T7, L5, Right and Left SI
- Extraspinal restrictions/subluxations: none noted
- Tissue Tone Changes: muscle spasms present, mild graded at hypertonic.
- Leg Length Check revealed short left leg with legs extended-patient prone, became even with legs in flexion with no cervical syndrome.
- Active Trigger Points: Mild

Radiographs:

- Rationale: Based upon the patient's history and examination, radiographs were ordered. As routine procedure the patient confirmed that there were no contraindications to taking radiographs, including but not limited to pregnancy, trying to become pregnant, receiving active radiation therapy, or other contraindication for X-Ray exposure. The rationale was due to need of structural integrity assessment and recent trauma.
- Views: The following radiographs were performed today in the standing (weight bearing) position with the following



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/24/2025 2:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

view(s): Cervical AP/L-OM, Cervical-Lateral, Cervical-Extension, Cervical-Forward Flexion, Thoracic-AP, Thoracic-Lateral, Lumbar-AP, Lumbar-Lateral, Right Knee-AP/Lateral.

Assessment:

The patient is of good health and is expected to make good progress and recovery with few residuals. Patient noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that their recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Based on the patient's history and my physical examination findings, it is my medical opinion that in all reasonable medical probability the patient's injuries were due to their accident.

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/24/2025 2:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

visits per week for next 2 weeks.

Therapeutic exercise will begin in 2-3 weeks once the patient has reached the sub-acute phase of care. The goals for these exercises is to, stretch and strengthen the injured muscles, as well as to help diminish the build up of scar tissue.

Today's Treatment

ROF (10 MIN)

I have reviewed the examination findings with patient. We discussed the length of treatment (frequency of visits and duration of treatment plan). We went over the goals and projected outcomes for treatment. We discussed the therapies that will be used during this treatment and the benefits they provide. The patient understood the findings and the importance of complying with the ordered treatment plan. Treatment plan will begin as recommended.

PostureRay reports were created, reviewed and explained to the patient. This software analyzes spinal x-rays and plots abnormal deviations digitally.

- Primary Treatment: Diversified- Chiropractic Manipulative Therapy (CMT) to the following spinal level(s): C3, C4, C5, C6 T3, T5, T4, T6, T7 L3, L4, L5

- EMS Unattended low volt EMS pads were applied to posterior cervical (neck) and Thoracolumbar region region(s) for 15 minutes. The goal for this therapy is to: relieve pain, decrease muscle spasms, reduce edema, promote tissue healing

-Hot/Cold Pack: dry hot/cold pack applied to posterior cervical (neck) and Thoracolumbar region region(s) for 15 minutes.

- Tx Effect: Treatment rendered, Examination performed without incident and responding as expected.

- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/24/2025 2:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

- Advised: Cold facilitates healing and symptom management. Home cold pack was dispensed and advised to use per instructions.

- Advised: A TENS unit was given to the patient today. The patient was instructed on how to use the unit, as well as how long to use the unit for. (See Letter of Medical Necessity)

Kyara Veron has been referred out for an MRI to rule out or confirm our diagnosis.

Kyara Veron has been referred out to Pain Management for further evaluation to rule out or confirm our diagnosis.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/25/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #2**
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has stayed the same since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has stayed the same with this complaint. 6-7/10 reported using the Visual Analog Scale. She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient feels pain in her shoulder blades and lower back. Patient did not take any medication.

Objective:

The patient demonstrated antalgic movement with guarding. ROM were restricted in the neck and low back areas. The patient was able to tolerate today's treatment with no increase in symptoms.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/25/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

- EMS Unattended low volt EMS pads were applied to lumbar, cervical region(s) for 15 minutes. The goal for this therapy is to: relieve pain, decrease muscle spasms, reduce edema, promote tissue healing

-Hot/Cold Pack: dry hot/cold pack applied to lumbar region(s) for 15 minutes.

- Tx Effect: Treatment rendered without incident and responding as expected.

- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/26/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #3
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has stayed the same since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has stayed the same with this complaint. 5/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient feels pain in between her shoulder blades and has not taken any medication.

Objective:

The patient demonstrated antalgic movement with guarding. ROM were restricted in the neck and low back areas.

The patient was able to tolerate today's treatment with no increase in symptoms.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:

The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/26/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:
- EMS Unattended low volt EMS pads were applied to lumbar, cervical region(s) for 15 minutes. The goal for this therapy is to: relieve pain, decrease muscle spasms, reduce edema, promote tissue healing
- Hot/Cold Pack: dry hot/cold pack applied to lumbar region(s) for 15 minutes.
- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/27/2025 9:45AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #4
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 4/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient still continues to feel pain in between her shoulder blades and lower back.

Objective:

The patient demonstrated gradual improvement in movement without guarding. ROM is increasing in the neck and low back areas. The patient was able to tolerate today's treatment with no increase in symptoms.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:

The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/27/2025 9:45AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:
- EMS Unattended low volt EMS pads were applied to lumbar, cervical region(s) for 15 minutes. The goal for this therapy is to: relieve pain, decrease muscle spasms, reduce edema, promote tissue healing
- Hot/Cold Pack: dry hot/cold pack applied to lumbar region(s) for 15 minutes.
- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/28/2025 6:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #5
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has worsened since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has worsened with this complaint. 5-6/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient continues to feel pain in between shoulders, patient has not taken medication for pain.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:

The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/28/2025 6:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
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Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Lateral Neck Stretch: Start by looking straight ahead. Slowly lean your head to the left. Using your left hand for resistance, use the muscles in your neck to press against it. Hold for 15-30 seconds, then return to the starting position. Then, slowly lean your head to the other side. Hold for 15-30 seconds. Return to the starting position.

Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Chin Tucks: Start by keeping your neck and shoulders in a relaxed position. Looked straight ahead. Then, gently glide your chin straight back – tuck your chin to your neck area by not holding your breath. You will feel a little pull around your upper neck. Remember also not to move your head up or down or bending your neck forward. Hold that



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/28/2025 6:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

position for about 5 seconds. Bring your chin forward again. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Cat-Cow: Kneel on the floor and put your hands on the floor in front of you. Keep your hands shoulder-width apart and your knees directly below your hips. Inhale deeply while curving your lower back and bringing your head up, tilting your pelvis up like a "cow." Exhale deeply and bring your abdomen in, arching your spine and bringing your head and pelvis down like a "cat." Repeat several times. Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Bird-Dog: Begin on all fours in the tabletop position. Place your knees under your hips and your hands under your shoulders. Maintain a neutral spine by engaging your abdominal muscles. Draw your shoulder blades together. Raise your right arm and left leg, keeping your shoulders and hips parallel to the floor. Lengthen the back of your neck and tuck your chin into your chest to gaze down at the floor. Hold this position for a few seconds, then lower back down to the starting position. Raise your left arm and right leg, holding this position for a few seconds. Return to the starting position. Muscle(s) abdominal muscles, erector spinae, Core and Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Bridges: Lie face up on the floor, with your knees bent and feet flat on the ground. Keep your arms at your side with your palms down. Lift your hips off the ground until your knees, hips and shoulders form a straight line. Squeeze those glutes hard and keep your abs drawn in so you don't overextend your back during the exercise. Hold your bridged position for a couple of seconds before easing back down. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Mckinze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 02/28/2025 6:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

Thread the Needle: Begin on the floor in a quadruped position (hand and knees). Hands should be placed directly under shoulders with hips above knees. Reach under, and across the body, stretching back of shoulder and upper back. Hold for 20-30 seconds. Return to start position. Repeat prescribed repetitions and sets. Alternate sides as directed. Muscle(s) latissimus dorsi, erector spinae, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.

- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/03/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #6**
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 4/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states she feels most pain in her lower back. She feels sore in between her shoulders and would rate the soreness a 3/10.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/03/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Lateral Neck Stretch: Start by looking straight ahead. Slowly lean your head to the left. Using your left hand for resistance, use the muscles in your neck to press against it. Hold for 15-30 seconds, then return to the starting position. Then, slowly lean your head to the other side. Hold for 15-30 seconds. Return to the starting position.

Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Chin Tucks: Start by keeping your neck and shoulders in a relaxed position. Looked straight ahead. Then, gently glide your chin straight back – tuck your chin to your neck area by not holding your breath. You will feel a little pull around



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/03/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

your upper neck. Remember also not to move your head up or down or bending your neck forward. Hold that position for about 5 seconds. Bring your chin forward again. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Cat-Cow: Kneel on the floor and put your hands on the floor in front of you. Keep your hands shoulder-width apart and your knees directly below your hips. Inhale deeply while curving your lower back and bringing your head up, tilting your pelvis up like a "cow." Exhale deeply and bring your abdomen in, arching your spine and bringing your head and pelvis down like a "cat." Repeat several times. Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Bird-Dog: Begin on all fours in the tabletop position. Place your knees under your hips and your hands under your shoulders. Maintain a neutral spine by engaging your abdominal muscles. Draw your shoulder blades together. Raise your right arm and left leg, keeping your shoulders and hips parallel to the floor. Lengthen the back of your neck and tuck your chin into your chest to gaze down at the floor. Hold this position for a few seconds, then lower back down to the starting position. Raise your left arm and right leg, holding this position for a few seconds. Return to the starting position. Muscle(s) abdominal muscles, erector spinae, Core and Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Bridges: Lie face up on the floor, with your knees bent and feet flat on the ground. Keep your arms at your side with your palms down. Lift your hips off the ground until your knees, hips and shoulders form a straight line. Squeeze those glutes hard and keep your abs drawn in so you don't overextend your back during the exercise. Hold your bridged position for a couple of seconds before easing back down. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Mckinze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/03/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

Therapeutic Exercise Group 4 #4 - 8 min

Thread the Needle: Begin on the floor in a quadruped position (hand and knees). Hands should be placed directly under shoulders with hips above knees. Reach under, and across the body, stretching back of shoulder and upper back. Hold for 20-30 seconds. Return to start position. Repeat prescribed repetitions and sets. Alternate sides as directed. Muscle(s) latissimus dorsi, erector spinae, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/04/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #7
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has worsened since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has worsened with this complaint. 5-6/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states she feels pain in between her shoulder blades.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:

The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/04/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Lateral Neck Stretch: Start by looking straight ahead. Slowly lean your head to the left. Using your left hand for resistance, use the muscles in your neck to press against it. Hold for 15-30 seconds, then return to the starting position. Then, slowly lean your head to the other side. Hold for 15-30 seconds. Return to the starting position.

Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Chin Tucks: Start by keeping your neck and shoulders in a relaxed position. Looked straight ahead. Then, gently glide your chin straight back – tuck your chin to your neck area by not holding your breath. You will feel a little pull around your upper neck. Remember also not to move your head up or down or bending your neck forward. Hold that



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/04/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

position for about 5 seconds. Bring your chin forward again. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Cat-Cow: Kneel on the floor and put your hands on the floor in front of you. Keep your hands shoulder-width apart and your knees directly below your hips. Inhale deeply while curving your lower back and bringing your head up, tilting your pelvis up like a "cow." Exhale deeply and bring your abdomen in, arching your spine and bringing your head and pelvis down like a "cat." Repeat several times. Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Bird-Dog: Begin on all fours in the tabletop position. Place your knees under your hips and your hands under your shoulders. Maintain a neutral spine by engaging your abdominal muscles. Draw your shoulder blades together. Raise your right arm and left leg, keeping your shoulders and hips parallel to the floor. Lengthen the back of your neck and tuck your chin into your chest to gaze down at the floor. Hold this position for a few seconds, then lower back down to the starting position. Raise your left arm and right leg, holding this position for a few seconds. Return to the starting position. Muscle(s) abdominal muscles, erector spinae, Core and Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Bridges: Lie face up on the floor, with your knees bent and feet flat on the ground. Keep your arms at your side with your palms down. Lift your hips off the ground until your knees, hips and shoulders form a straight line. Squeeze those glutes hard and keep your abs drawn in so you don't overextend your back during the exercise. Hold your bridged position for a couple of seconds before easing back down. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Mckinze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/04/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

Thread the Needle: Begin on the floor in a quadruped position (hand and knees). Hands should be placed directly under shoulders with hips above knees. Reach under, and across the body, stretching back of shoulder and upper back. Hold for 20-30 seconds. Return to start position. Repeat prescribed repetitions and sets. Alternate sides as directed. Muscle(s) latissimus dorsi, erector spinae, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.

- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/05/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #8**
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has stayed the same since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has stayed the same with this complaint. 5-6/10 reported using the Visual Analog Scale. She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states she feels pain in between her shoulder blades rating about a 5 but that she feels most pain in her lower back.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/05/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Lateral Neck Stretch: Start by looking straight ahead. Slowly lean your head to the left. Using your left hand for resistance, use the muscles in your neck to press against it. Hold for 15-30 seconds, then return to the starting position. Then, slowly lean your head to the other side. Hold for 15-30 seconds. Return to the starting position.

Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Chin Tucks: Start by keeping your neck and shoulders in a relaxed position. Looked straight ahead. Then, gently glide your chin straight back – tuck your chin to your neck area by not holding your breath. You will feel a little pull around



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/05/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

your upper neck. Remember also not to move your head up or down or bending your neck forward. Hold that position for about 5 seconds. Bring your chin forward again. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Cat-Cow: Kneel on the floor and put your hands on the floor in front of you. Keep your hands shoulder-width apart and your knees directly below your hips. Inhale deeply while curving your lower back and bringing your head up, tilting your pelvis up like a "cow." Exhale deeply and bring your abdomen in, arching your spine and bringing your head and pelvis down like a "cat." Repeat several times. Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Bird-Dog: Begin on all fours in the tabletop position. Place your knees under your hips and your hands under your shoulders. Maintain a neutral spine by engaging your abdominal muscles. Draw your shoulder blades together. Raise your right arm and left leg, keeping your shoulders and hips parallel to the floor. Lengthen the back of your neck and tuck your chin into your chest to gaze down at the floor. Hold this position for a few seconds, then lower back down to the starting position. Raise your left arm and right leg, holding this position for a few seconds. Return to the starting position. Muscle(s) abdominal muscles, erector spinae, Core and Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Bridges: Lie face up on the floor, with your knees bent and feet flat on the ground. Keep your arms at your side with your palms down. Lift your hips off the ground until your knees, hips and shoulders form a straight line. Squeeze those glutes hard and keep your abs drawn in so you don't overextend your back during the exercise. Hold your bridged position for a couple of seconds before easing back down. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Mckinze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/05/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

Therapeutic Exercise Group 4 #4 - 8 min

Thread the Needle: Begin on the floor in a quadruped position (hand and knees). Hands should be placed directly under shoulders with hips above knees. Reach under, and across the body, stretching back of shoulder and upper back. Hold for 20-30 seconds. Return to start position. Repeat prescribed repetitions and sets. Alternate sides as directed. Muscle(s) latissimus dorsi, erector spinae, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/06/2025 3:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #9**
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 4/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states the pain in between her shoulders is getting better.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/06/2025 3:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Lateral Neck Stretch: Start by looking straight ahead. Slowly lean your head to the left. Using your left hand for resistance, use the muscles in your neck to press against it. Hold for 15-30 seconds, then return to the starting position. Then, slowly lean your head to the other side. Hold for 15-30 seconds. Return to the starting position.

Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Chin Tucks: Start by keeping your neck and shoulders in a relaxed position. Looked straight ahead. Then, gently glide your chin straight back – tuck your chin to your neck area by not holding your breath. You will feel a little pull around



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/06/2025 3:30PM

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your upper neck. Remember also not to move your head up or down or bending your neck forward. Hold that position for about 5 seconds. Bring your chin forward again. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Cat-Cow: Kneel on the floor and put your hands on the floor in front of you. Keep your hands shoulder-width apart and your knees directly below your hips. Inhale deeply while curving your lower back and bringing your head up, tilting your pelvis up like a "cow." Exhale deeply and bring your abdomen in, arching your spine and bringing your head and pelvis down like a "cat." Repeat several times. Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Bird-Dog: Begin on all fours in the tabletop position. Place your knees under your hips and your hands under your shoulders. Maintain a neutral spine by engaging your abdominal muscles. Draw your shoulder blades together. Raise your right arm and left leg, keeping your shoulders and hips parallel to the floor. Lengthen the back of your neck and tuck your chin into your chest to gaze down at the floor. Hold this position for a few seconds, then lower back down to the starting position. Raise your left arm and right leg, holding this position for a few seconds. Return to the starting position. Muscle(s) abdominal muscles, erector spinae, Core and Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Bridges: Lie face up on the floor, with your knees bent and feet flat on the ground. Keep your arms at your side with your palms down. Lift your hips off the ground until your knees, hips and shoulders form a straight line. Squeeze those glutes hard and keep your abs drawn in so you don't overextend your back during the exercise. Hold your bridged position for a couple of seconds before easing back down. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Mckinze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins



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Therapeutic Exercise Group 4 #4 - 8 min

Thread the Needle: Begin on the floor in a quadruped position (hand and knees). Hands should be placed directly under shoulders with hips above knees. Reach under, and across the body, stretching back of shoulder and upper back. Hold for 20-30 seconds. Return to start position. Repeat prescribed repetitions and sets. Alternate sides as directed. Muscle(s) latissimus dorsi, erector spinae, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/07/2025 3:30PM

Chart: VEKY000001

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Daily Encounter:

Treatment for acute/active care on **Visit #10**

Office Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 3/10 reported using the Visual Analog Scale.

The pain was described as achey, dull, soreness, stiffness.

She reports her ability to participate with sitting, moving head, bending over at the waist with an increase in pain.

Daily Encounter: treatment for acute/active care on 03/07/2025

RE-EVALUATION:

Kyara returned today for a periodic re-evaluation of the condition currently being treated for the injuries sustained due to their accident. The findings are as follows:

Objective:

Musculoskeletal - Range of Motion - Knee - Active:

-Flexion: 130 /130 degrees w/o pain

-Hyperextension: 0/0 to 15 degrees w/o pain **Ortho-** Cervical Distraction Test performed. Patient indicated no pain.

Ortho- Shoulder Depression Test performed. Patient indicated pain bilaterally. **Ortho-** Maximal Compression Test performed. Patient indicated pain bilaterally. Ortho-Straight Leg Raiser Test performed. Patient indicated pain bilaterally.

Ortho-Kemp's Test performed. Patient indicated pain bilaterally. **Ortho-**Knee Distraction Test performed.

Patient indicated local pain right side. Musculoskeletal - Range of Motion - Cervical -

-Flexion: 55/60 degrees

-Extension: 50/55 degrees

-Left Lat. Flexion: 35/40 degrees

-Right Lat. Flexion: 35/40 degrees

-Left Rotation: 75/80 degrees

-Right Rotation: 75/80 degrees



Q Factor Chiropractic

Patient: Kyara Veron	DOB: 02/23/2004	Sex: F
Provider: Dr. Edward Amadi, DC	Visit: 03/07/2025 3:30PM	Chart: VEKY000001
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Musculoskeletal - Range of Motion - Thoraco-Lumbar - Active

- Flexion: 85/90 degrees
- Extension: 25/30 degrees
- Left Lat. Flexion: 30/35 degrees
- Right Lat. Flexion: 25/30 degrees
- Left Rotation: 30/35 degrees
- Right Rotation: 30/35 degrees

Neuro-Sensory-Pain: Evaluations performed bilaterally. Dermatomal normal findings at all upper & lower spinal segments.

Neuro-Light Touch: Evaluations performed and indicated light touch sensations were within normal limits at all upper & lower spinal segments.

Neuro-Deep Tendon Reflexes (normal 2+):

- Biceps: Left 2+, Right 2+,
- Triceps: Left 2+, Right 2+,
- Brachioradialis: Left 2+, Right 2+,
- Patellar: Left 2+, Right 2+,
- Achilles: Left 2+, Right 2+,

Neuro-Upper extremity resistive isometric motor testing (normal 5/5):

- Shoulder Elevation: Left: 5 / 5 Right: 5 / 5.
- Deltoid: Left: 5 / 5 Right: 5 / 5
- Biceps: Left: 5 / 5 Right: 5 / 5
- Triceps: Left: 5 / 5 Right: 5 / 5
- Wrist Flexors: Left: 5 / 5 Right: 5 / 5
- Wrist Extensors: Left: 5 / 5 Right: 5 / 5
- Finger Extensors: Left: 5 / 5 Right: 5 / 5



Q Factor Chiropractic

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Provider: Dr. Edward Amadi, DC

Visit: 03/07/2025 3:30PM

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-
- Finger Flexors: Left: 5 / 5 Right: 5 / 5
 - Finger Abductors: Left: 5 / 5 Right: 5 / 5
 - Palmar Interossei: Left: 5 / 5 Right: 5 / 5

Neuro-Lower extremity resistive isometric motor testing (normal 5/5):

- Iliopsoas: Left: 5 / 5 Right: 5 / 5
- Quadriceps: Left: 5 / 5 Right: 5 / 5
- Anterior Tibialis: Left: 5 / 5 Right: 5 / 5
- Hallucis Longus: Left: 5 / 5 Right: 5 / 5
- Ext Digitorum Longus & Brevis: Left: 5 / 5 Right: 5 / 5
- Gluteus Medius: Left: 5 / 5 Right: 5 / 5

Musculoskeletal

- Gait and Station: normal gait and normal balance
- Inspection/Percussion +/- Palpation: cervical erector paraspinal, thoracic and lumbar erector paraspinal, piriformis bilateral
- Postural Analysis: Short right leg (pelvic deficiency)
- Spinal Stability/Restriction(s)/Subluxation(s): C5, C7-T1, T5-T7, L5, Right and Left SI
- Extrapinal restrictions/subluxations: none noted
- Tissue Tone Changes: muscle spasms present, mild graded at hypertonic.
- Leg Length Check revealed short right leg with legs extended-patient prone, became even with legs in flexion with no cervical syndrome.
- Active Trigger Points: Mild

Assessment:

Kyara Veron feels the complaint has improved their ability to standing/walking/running and has changed about **30%** since the onset of this complaint/condition. In consideration of the findings from today's re-evaluation of Kyara Veron's complaints, the course of care and my examination findings for this condition, continued treatment is necessary and the new treatment plan will be continued.



Q Factor Chiropractic

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Provider: Dr. Edward Amadi, DC

Visit: 03/07/2025 3:30PM

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Prognosis:
The prognosis for this patient is as follows: Good

Diagnosis:
Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

ONGOING TREATMENT PLAN:

Kyara Veron treatment plan for this episode began on date of accident as noted on initial exam and will be evaluated in 7-9 visits to determined continuing treatment.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

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- Short Term Tx Goal: To increase ability to perform activities of daily living exacerbated by the accident as noted on the initial exam the re-exam date within 7-9 visits unless improvement warrants discharge sooner.

- Long Term Goal: Attain pre-condition/pre-injury status

- Tx Effectiveness: Overall effectiveness of treatment for this complaint will be evaluated by analyzing the no FOA performed functional outcome assessment tools. The goal score is 10% or better.

- Primary Treatment: Diversified- Chiropractic Manipulative Therapy (CMT) (approx: 8 to 16 visits) to the cervical dorsal spinal region, lumbosacral spinal region and thoraco-lumbar spinal region at a frequency and duration of 3 visits per week for next 3 weeks and 2 visits per week for next 4 weeks followed by a re-exam within 7-9 visits.

- Supportive Therapy: To optimize the treatment effectiveness, the following supportive therapy(ies) are ordered:

Therapeutic exercise will continue in the sub-acute phase of care. The goal for these exercises is to stretch and strengthen the injured muscles, as well as to help diminish the buildup of scar tissue.

Today's Treatment

- Primary Treatment: Diversified- Chiropractic Manipulative Therapy (CMT) to the following spinal level(s): C3, C4, C5, C6 T3, T5, T4, T6, T7 L3, L4, L5

- Tx Effect: Treatment rendered, Examination performed without incident and responding as expected.

- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/10/2025 3:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #11
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has worsened since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has worsened with this complaint. 5/10 reported using the Visual Analog Scale. She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states the pain in her mid-back is feeling better at a 3/10 but that her lower back is in pain rating it a 5/10.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/10/2025 3:00PM

Chart: VEKY000001

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Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

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Provider: Dr. Edward Amadi, DC

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your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position



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Patient: Kyara Veron

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and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/11/2025 3:00PM

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Office: Q Factor Chiropractic

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Daily Encounter:

Treatment for acute/active care on Visit #12
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 4/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states that she feels a lot better but cannot bend down too fast or it will hurt.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/11/2025 3:00PM

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Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck



Q Factor Chiropractic

Patient: Kyara Veron

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75039

lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the



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75039

opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.

- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/12/2025 4:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #13
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 3/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states that she feels slight pain in her lower back.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/12/2025 4:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/12/2025 4:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/12/2025 4:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
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opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/13/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #14
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has stayed the same since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has stayed the same with this complaint. 3/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states that she feels pain in her lower back.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/13/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/13/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/13/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/14/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #15
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has stayed the same since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has stayed the same with this complaint. 3/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states that she feels sore and most of her pain is in her lower back.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/14/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/14/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/14/2025 11:00AM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.

- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/17/2025 4:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #16
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 2/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient feels slight pain in her lower back.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/17/2025 4:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/17/2025 4:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/17/2025 4:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/18/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #17
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has worsened since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has worsened with this complaint. 3-4/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient feels pain in her mid-back and lower back.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:

The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/18/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/18/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/18/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/19/2025 12:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #18
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has worsened since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has worsened with this complaint. 5/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient is feeling pain in her lower back at a 5. She states her mid back is at a 3/10 and has not taken any medication.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:

The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/19/2025 12:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/19/2025 12:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/19/2025 12:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/20/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #19**
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has stayed the same since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has stayed the same with this complaint. 5/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient is feeling the same way as yesterday. Patient did not take any medication.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:

The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/20/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/20/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Spine Roll down: Stand with your feet hips width apart. Breathe in and roll your head and upper chest downwards from the top of your spine by bringing your chin towards your chest opening your neck. Curl into yourself, gathering your tummy allow your arms and shoulder blades to hang freely on either side. When your hands are around knee level, take a breath in. As your breath out, gather your tummy in. Roll your body back up, pulling from the lower back muscles. Let your shoulders hang loosely to the side through the entire movement. At the end of the movement unfold the neck to and bring your shoulder blades back to return to the start position Muscle(s) thoracic cage, erector spinae, Upper- Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep you legs flat on floor up to table-top position with your hips at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Child Pose: Get onto your hands and knees, and drop your buttocks back onto your heels. Stretch your hands forwards, dropping your head between your shoulders towards the floor. You will feel this stretch through your back and upper arms. Muscle(s) Gluteal muscles, piriformis, hamstrings, erector spinae, Gluteal muscles Low Back. 8 mins

Lower trunk rotation: Lie on your back in the supine position with your legs bent and your feet flat on the floor. Extend your arms at a 45 degree angle to your trunk with your palms facing downwards. Raise your head just off the floor and keep your chin tucked in. Maintain contact between your knees and feet, and tighten your abdominal muscles for the duration of the exercise. Now rotate your legs to the left to the limit of your ability, make sure the opposite shoulder remains in contact with the floor throughout the movement. Move back to the neutral position



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/20/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
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and repeat, alternating sides for the desired number of repetitions. Do not rotate the legs to a point beyond which the arms and shoulders can no longer maintain contact with the floor. Perform this exercise in a slow controlled, manner within the range of your mobility. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze: Lie down on your stomach. Prop yourself up on your forearms with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/21/2025 3:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #20**

Office Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 2/10 reported using the Visual Analog Scale.

The pain was described as achey, dull, soreness, stiffness.

She reports her ability to participate with sitting, moving head, bending over at the waist with an increase in pain.

Daily Encounter: treatment for acute/active care on 03/21/2025

RE-EVALUATION:

Kyara returned today for a periodic re-evaluation of the condition currently being treated for the injuries sustained due to their accident. The findings are as follows:

Objective:

Musculoskeletal - Range of Motion - Knee - Active:

-Flexion: 130 /130 degrees w/o pain

-Hyperextension: 0/0 to 15 degrees w/o pain **Ortho-** Cervical Distraction Test performed. Patient indicated no pain.

Ortho- Shoulder Depression Test performed. Patient indicated pain bilaterally. **Ortho-** Maximal Compression Test performed. Patient indicated pain bilaterally. Ortho-Straight Leg Raiser Test performed. Patient indicated pain bilaterally.

Ortho-Kemp's Test performed. Patient indicated pain bilaterally. **Ortho-**Knee Distraction Test performed.

Patient indicated local pain right side. Musculoskeletal - Range of Motion - Cervical -

-Flexion: 55/60 degrees

-Extension: 50/55 degrees

-Left Lat. Flexion: 35/40 degrees

-Right Lat. Flexion: 35/40 degrees

-Left Rotation: 75/80 degrees

-Right Rotation: 75/80 degrees



Q Factor Chiropractic

Patient: Kyara Veron	DOB: 02/23/2004	Sex: F
Provider: Dr. Edward Amadi, DC	Visit: 03/21/2025 3:00PM	Chart: VEKY000001
Office: Q Factor Chiropractic	Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039	

Musculoskeletal - Range of Motion - Thoraco-Lumbar - Active

- Flexion: 85/90 degrees
- Extension: 25/30 degrees
- Left Lat. Flexion: 30/35 degrees
- Right Lat. Flexion: 25/30 degrees
- Left Rotation: 30/35 degrees
- Right Rotation: 30/35 degrees

Neuro-Sensory-Pain: Evaluations performed bilaterally. Dermatomal normal findings at all upper & lower spinal segments.

Neuro-Light Touch: Evaluations performed and indicated light touch sensations were within normal limits at all upper & lower spinal segments.

Neuro-Deep Tendon Reflexes (normal 2+):

- Biceps: Left 2+, Right 2+,
- Triceps: Left 2+, Right 2+,
- Brachioradialis: Left 2+, Right 2+,
- Patellar: Left 2+, Right 2+,
- Achilles: Left 2+, Right 2+,

Neuro-Upper extremity resistive isometric motor testing (normal 5/5):

- Shoulder Elevation: Left: 5 / 5 Right: 5 / 5.
 - Deltoid: Left: 5 / 5 Right: 5 / 5
 - Biceps: Left: 5 / 5 Right: 5 / 5
 - Triceps: Left: 5 / 5 Right: 5 / 5
 - Wrist Flexors: Left: 5 / 5 Right: 5 / 5
 - Wrist Extensors: Left: 5 / 5 Right: 5 / 5
 - Finger Extensors: Left: 5 / 5 Right: 5 / 5
-



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/21/2025 3:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

-
- Finger Flexors: Left: 5 / 5 Right: 5 / 5
 - Finger Abductors: Left: 5 / 5 Right: 5 / 5
 - Palmar Interossei: Left: 5 / 5 Right: 5 / 5

Neuro-Lower extremity resistive isometric motor testing (normal 5/5):

- Iliopsoas: Left: 5 / 5 Right: 5 / 5
- Quadriceps: Left: 5 / 5 Right: 5 / 5
- Anterior Tibialis: Left: 5 / 5 Right: 5 / 5
- Hallucis Longus: Left: 5 / 5 Right: 5 / 5
- Ext Digitorum Longus & Brevis: Left: 5 / 5 Right: 5 / 5
- Gluteus Medius: Left: 5 / 5 Right: 5 / 5

Musculoskeletal

- Gait and Station: normal gait and normal balance
- Inspection/Percussion +/- Palpation: cervical erector paraspinal, thoracic and lumbar erector paraspinal, piriformis bilateral
- Postural Analysis: Short right leg (pelvic deficiency)
- Spinal Stability/Restriction(s)/Subluxation(s): C5, C7-T1, T5-T7, L5, Right and Left SI
- Extrapinal restrictions/subluxations: none noted
- Tissue Tone Changes: muscle spasms present, mild graded at hypertonic.
- Leg Length Check revealed short right leg with legs extended-patient prone, became even with legs in flexion with no cervical syndrome.
- Active Trigger Points: Mild

Assessment:

Kyara Veron feels the complaint has improved their ability to standing/walking/running and has changed about **30%** since the onset of this complaint/condition. In consideration of the findings from today's re-evaluation of Kyara Veron's complaints, the course of care and my examination findings for this condition, continued treatment is necessary and the new treatment plan will be continued.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/21/2025 3:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Prognosis:
The prognosis for this patient is as follows: Good

Diagnosis:
Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:
ONGOING TREATMENT PLAN:
Kyara Veron treatment plan for this episode began on date of accident as noted on initial exam and will be evaluated in 7-9 visits to determined continuing treatment.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/21/2025 3:00PM

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- Short Term Tx Goal: To increase ability to perform activities of daily living exacerbated by the accident as noted on the initial exam the re-exam date within 7-9 visits unless improvement warrants discharge sooner.

- Long Term Goal: Attain pre-condition/pre-injury status

- Tx Effectiveness: Overall effectiveness of treatment for this complaint will be evaluated by analyzing the no FOA performed functional outcome assessment tools. The goal score is 10% or better.

- Primary Treatment: Diversified- Chiropractic Manipulative Therapy (CMT) (approx: 8 to 16 visits) to the cervical dorsal spinal region, lumbosacral spinal region and thoraco-lumbar spinal region at a frequency and duration of 3 visits per week for next 3 weeks and 2 visits per week for next 4 weeks followed by a re-exam within 7-9 visits.

- Supportive Therapy: To optimize the treatment effectiveness, the following supportive therapy(ies) are ordered:

Therapeutic exercise will continue in the sub-acute phase of care. The goal for these exercises is to stretch and strengthen the injured muscles, as well as to help diminish the buildup of scar tissue.

Today's Treatment

- Primary Treatment: Diversified- Chiropractic Manipulative Therapy (CMT) to the following spinal level(s): C3, C4, C5, C6 T3, T5, T4, T6, T7 L3, L4, L5

- Tx Effect: Treatment rendered, Examination performed without incident and responding as expected.

- Next Visit: continue with treatment plan as scheduled

Kyara Veron has viewed MRI with doctor to confirm diagnosis. MRI came out positive.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/24/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #21**
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has worsened since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has worsened with this complaint. 6/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient feels most pain in her lower back but states that her neck feels sore.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/24/2025 4:00PM

Chart: VEKY000001

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Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/24/2025 4:00PM

Chart: VEKY000001

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your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Windmill trunk rotation: Start position is standing in a wide stance with the arms stretched out to the side at shoulder level height. Move into a semi-squat and hold when arms can touch the floor, then rotate at the hips and the trunk to reach the arms toward the opposite foot. Stay balanced, with slightly more weight on the side of the reach and keep the head and chest up and aligned. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep your legs up to table-top position with your hips and knee at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Crunches: Lie on your back with your knees bent and your feet flat on the floor. Stretch your arms out on the floor behind you, and tuck your chin to your chest. Reach your hands back up and over to touch behind your thighs, curling your upper body off the floor. Control the movement back down, keeping your chin tucked in. Repeat Muscle(s) erector spinae, Core muscles, Core. 8 mins

Core Stabilization : Lie on your front with your head in a neutral position. Extend your arms straight out in front of you. Simultaneously lift one arm and the opposite leg, clenching your buttock muscles and your abdominal muscles to maintain stability in your body. Keep a neutral alignment with your head throughout the movement. Lower the



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/24/2025 4:00PM

Chart: VEKY000001

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arm and leg together, then repeat with the other pair. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze phase 2 : Lie down on your stomach. Prop yourself up on your hands with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/25/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #22
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 4/10 reported using the Visual Analog Scale. She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states she is experiencing soreness and would rate her neck pain a 0, her mid back a 2 and her lower back a 4.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/25/2025 4:00PM

Chart: VEKY000001

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Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/25/2025 4:00PM

Chart: VEKY000001

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lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Windmill trunk rotation: Start position is standing in a wide stance with the arms stretched out to the side at shoulder level height. Move into a semi-squat and hold when arms can touch the floor, then rotate at the hips and the trunk to reach the arms toward the opposite foot. Stay balanced, with slightly more weight on the side of the reach and keep the head and chest up and aligned. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep your legs up to table-top position with your hips and knee at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Crunches: Lie on your back with your knees bent and your feet flat on the floor. Stretch your arms out on the floor behind you, and tuck your chin to your chest. Reach your hands back up and over to touch behind your thighs, curling your upper body off the floor. Control the movement back down, keeping your chin tucked in. Repeat Muscle(s) erector spinae, Core muscles, Core. 8 mins

Core Stabilization : Lie on your front with your head in a neutral position. Extend your arms straight out in front of you. Simultaneously lift one arm and the opposite leg, clenching your buttock muscles and your abdominal muscles



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/25/2025 4:00PM

Chart: VEKY000001

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to maintain stability in your body. Keep a neutral alignment with your head throughout the movement. Lower the arm and leg together, then repeat with the other pair. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze phase 2 : Lie down on your stomach. Prop yourself up on your hands with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/26/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on Visit #23
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has worsened since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has worsened with this complaint. 6-7/10 reported using the Visual Analog Scale.

She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient is in a lot of pain because of work. She states she feels most pain in her lower left side of back.

Objective:

The patient had discomfort with exercises and stretches today and showed mild to moderate guarding. ROM is restricted. The patient was able to tolerate exercise and complete today's session without incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:

The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/26/2025 4:00PM

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Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

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your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Windmill trunk rotation: Start position is standing in a wide stance with the arms stretched out to the side at shoulder level height. Move into a semi-squat and hold when arms can touch the floor, then rotate at the hips and the trunk to reach the arms toward the opposite foot. Stay balanced, with slightly more weight on the side of the reach and keep the head and chest up and aligned. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep your legs up to table-top position with your hips and knee at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Crunches: Lie on your back with your knees bent and your feet flat on the floor. Stretch your arms out on the floor behind you, and tuck your chin to your chest. Reach your hands back up and over to touch behind your thighs, curling your upper body off the floor. Control the movement back down, keeping your chin tucked in. Repeat Muscle(s) erector spinae, Core muscles, Core. 8 mins

Core Stabilization : Lie on your front with your head in a neutral position. Extend your arms straight out in front of you. Simultaneously lift one arm and the opposite leg, clenching your buttock muscles and your abdominal muscles to maintain stability in your body. Keep a neutral alignment with your head throughout the movement. Lower the



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/26/2025 4:00PM

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arm and leg together, then repeat with the other pair. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze phase 2 : Lie down on your stomach. Prop yourself up on your hands with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/27/2025 4:00PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #24**
TeleHealth Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 4/10 reported using the Visual Analog Scale. She reports her ability to participate with working, reaching above head, moving head with an increase in pain. The pain was described as a dull ache.

Patient states she feels most pain in her lower back at a 4/10 but that her neck is a 0. She continues to feel slight pain in her mid-back at a 3.

Objective:

The patient stated the exercises are relieving the tightness and tension. The patient displayed an increase in ROM during exercises today. There was limited guarding with exercises performed today. The patient was able to tolerate today's treatment without further incident.

Assessment:

Kyara Veron is of good health and is expected to make good progress and recovery with few residuals. She noted no contraindications to chiropractic care. Based on patient's history, examination, and no complicating factors it is reasonable to believe that her recovery may take about the same length of time as an average patient with an uncomplicated case.

Prognosis:
The prognosis for this patient is as follows: Fair

Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.



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Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of lumbar region	10	M99.03		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of cervical spine, initial encounter	10	S13.4XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle, fascia and tendon at neck level, initial encounter	10	S16.1XXA		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of thoracic spine, initial encounter	10	S23.3XXA		active	Feb. 24, 2025, 2 p.m.
Strain of muscle and tendon of unspecified wall of thorax, initial encounter	10	S29.019A		active	Feb. 24, 2025, 2 p.m.
Sprain of ligaments of lumbar spine, initial encounter	10	S33.5XXA		active	Feb. 24, 2025, 2 p.m.
Unspecified injury of muscle, fascia and tendon of lower back, initial encounter	10	S39.002A		active	Feb. 24, 2025, 2 p.m.
Strain of unspecified muscle(s) and tendon(s) at lower leg level, right leg, initial encounter	10	S86.911A		active	Feb. 24, 2025, 2 p.m.
Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:

Today's Treatment:

- Supportive Therapy to optimize treatment effectiveness the following therapy(s) were performed today:

Therapeutic Exercise Group 1 #1 - 16 min

Levator stretch: Start in a seated position. Place the hand of the side you want to stretch behind your back. Take the opposite hand and pull your head forwards and to the opposite side at an angle, until you feel a stretch from the base of your skull down into your shoulder blade. Hold this stretch. Muscle(s) Trapezius, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins.

Chin Tucks w/ ROM: Sit or stand up straight. Ensure your posture is upright with your weight equally spread. From a neutral position with your head, pull your head backwards so that your chin tucks in. Feel the back of your neck



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lengthen. Keeping this position, tilt your head backwards to look up towards the ceiling. Ensure you continue with your chin tuck. Do not allow your shoulders to hunch up. Turn your head gently one way and then the other. Ensure you continue to attempt to move your head further back. Bring your head to the middle before returning back up to the starting position. Muscle(s) Paraspinals, Rhomboids, Levator scapulae, erector spinae, Neck and Mid- Back 8 mins

Therapeutic Exercise Group 2 #2 - 16 min

Windmill trunk rotation: Start position is standing in a wide stance with the arms stretched out to the side at shoulder level height. Move into a semi-squat and hold when arms can touch the floor, then rotate at the hips and the trunk to reach the arms toward the opposite foot. Stay balanced, with slightly more weight on the side of the reach and keep the head and chest up and aligned. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Dead Bug phase 2 : Lie on your back and keep your legs up to table-top position with your hips and knee at 90 degrees. Raise your arms straight up vertically over your head. Keeping your back flat, lower the opposite arm and leg away from one another towards the floor. Do not allow anything else to move and make sure your back stays flat on the floor. Return to the start position and repeat with the other pair. Muscle(s) thoracic cage, erector spinae, trapezius, deltoids Upper- Shoulders Mid Back Low Back. 8 mins

Therapeutic Exercise Group 3 #3 - 16 min

Crunches: Lie on your back with your knees bent and your feet flat on the floor. Stretch your arms out on the floor behind you, and tuck your chin to your chest. Reach your hands back up and over to touch behind your thighs, curling your upper body off the floor. Control the movement back down, keeping your chin tucked in. Repeat Muscle(s) erector spinae, Core muscles, Core. 8 mins

Core Stabilization : Lie on your front with your head in a neutral position. Extend your arms straight out in front of you. Simultaneously lift one arm and the opposite leg, clenching your buttock muscles and your abdominal muscles



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to maintain stability in your body. Keep a neutral alignment with your head throughout the movement. Lower the arm and leg together, then repeat with the other pair. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

Therapeutic Exercise Group 4 #4 - 8 min

Mckineze phase 2 : Lie down on your stomach. Prop yourself up on your hands with your shoulders above your elbows. Hold for 2 to 3 minutes. Lower your upper body. Repeat up to eight times a day. Muscle(s) latissimus dorsi, erector spinae, multifidi, quadratus lumborum, Low Back. 8 mins

- Tx Effect: Treatment rendered without incident and responding as expected.
- Next Visit: continue with treatment plan as scheduled



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/28/2025 3:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Daily Encounter:

Treatment for acute/active care on **Visit #25**

Office Visit

Subjective:

- Chief Complaint: Cervical, Thoracic, Lumbar, Knee (Right).

- Subjective/Patient Assessment: Kyara stated this complaint has improved since the last visit. She reports her ability to perform the ADL functions listed in their initial visit has improved with this complaint. 2/10 reported using the Visual Analog Scale.

The pain was described as achey, dull, soreness, stiffness.

She reports her ability to participate with sitting, moving head, bending over at the waist with an increase in pain.

Daily Encounter: treatment for acute/active care on 03/28/2025

RE-EVALUATION:

Kyara returned today for a periodic re-evaluation of the condition currently being treated for the injuries sustained due to their accident. The findings are as follows:

Objective:

Musculoskeletal - Range of Motion - Knee - Active:

-Flexion: 130 /130 degrees w/o pain

-Hyperextension: 0/0 to 15 degrees w/o pain **Ortho-** Cervical Distraction Test performed. Patient indicated no pain.

Ortho- Shoulder Depression Test performed. Patient indicated pain bilaterally. **Ortho-** Maximal Compression Test performed. Patient indicated pain bilaterally. Ortho-Straight Leg Raiser Test performed. Patient indicated pain bilaterally.

Ortho-Kemp's Test performed. Patient indicated pain bilaterally. **Ortho-**Knee Distraction Test performed.

Patient indicated local pain right side. Musculoskeletal - Range of Motion - Cervical -

-Flexion: 55/60 degrees

-Extension: 50/55 degrees

-Left Lat. Flexion: 35/40 degrees

-Right Lat. Flexion: 35/40 degrees

-Left Rotation: 75/80 degrees

-Right Rotation: 75/80 degrees



Q Factor Chiropractic

Patient: Kyara Veron	DOB: 02/23/2004	Sex: F
Provider: Dr. Edward Amadi, DC	Visit: 03/28/2025 3:30PM	Chart: VEKY000001
Office: Q Factor Chiropractic	Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039	

Musculoskeletal - Range of Motion - Thoraco-Lumbar - Active

- Flexion: 85/90 degrees
- Extension: 25/30 degrees
- Left Lat. Flexion: 30/35 degrees
- Right Lat. Flexion: 25/30 degrees
- Left Rotation: 30/35 degrees
- Right Rotation: 30/35 degrees

Neuro-Sensory-Pain: Evaluations performed bilaterally. Dermatomal normal findings at all upper & lower spinal segments.

Neuro-Light Touch: Evaluations performed and indicated light touch sensations were within normal limits at all upper & lower spinal segments.

Neuro-Deep Tendon Reflexes (normal 2+):

- Biceps: Left 2+, Right 2+,
- Triceps: Left 2+, Right 2+,
- Brachioradialis: Left 2+, Right 2+,
- Patellar: Left 2+, Right 2+,
- Achilles: Left 2+, Right 2+,

Neuro-Upper extremity resistive isometric motor testing (normal 5/5):

- Shoulder Elevation: Left: 5 / 5 Right: 5 / 5.
 - Deltoid: Left: 5 / 5 Right: 5 / 5
 - Biceps: Left: 5 / 5 Right: 5 / 5
 - Triceps: Left: 5 / 5 Right: 5 / 5
 - Wrist Flexors: Left: 5 / 5 Right: 5 / 5
 - Wrist Extensors: Left: 5 / 5 Right: 5 / 5
 - Finger Extensors: Left: 5 / 5 Right: 5 / 5
-



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

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-
- Finger Flexors: Left: 5 / 5 Right: 5 / 5
 - Finger Abductors: Left: 5 / 5 Right: 5 / 5
 - Palmar Interossei: Left: 5 / 5 Right: 5 / 5

Neuro-Lower extremity resistive isometric motor testing (normal 5/5):

- Iliopsoas: Left: 5 / 5 Right: 5 / 5
- Quadriceps: Left: 5 / 5 Right: 5 / 5
- Anterior Tibialis: Left: 5 / 5 Right: 5 / 5
- Hallucis Longus: Left: 5 / 5 Right: 5 / 5
- Ext Digitorum Longus & Brevis: Left: 5 / 5 Right: 5 / 5
- Gluteus Medius: Left: 5 / 5 Right: 5 / 5

Musculoskeletal

- Gait and Station: normal gait and normal balance
- Inspection/Percussion +/- Palpation: cervical erector paraspinal, thoracic and lumbar erector paraspinal, piriformis bilateral
- Postural Analysis: Short right leg (pelvic deficiency)
- Spinal Stability/Restriction(s)/Subluxation(s): C5, C7-T1, T5-T7, L5, Right and Left SI
- Extrapinal restrictions/subluxations: none noted
- Tissue Tone Changes: muscle spasms present, mild graded at hypertonic.
- Leg Length Check revealed short right leg with legs extended-patient prone, became even with legs in flexion with no cervical syndrome.
- Active Trigger Points: Mild

Assessment:

Kyara Veron feels the complaint has improved their ability to standing/walking/running and has changed about **95%** since the onset of this complaint/condition. In consideration of the findings from today's re-evaluation of Kyara Veron's complaints, the course of care and my examination findings for this condition, continued treatment is unnecessary and the new treatment plan will be discontinued, maximal medical improvement reached.



Q Factor Chiropractic

Patient: Kyara Veron

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Visit: 03/28/2025 3:30PM

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Address: 100 E. Royal Lane Suite 125, Irving, TX, 75039

Prognosis:
The prognosis for this patient is as follows: Good

Diagnosis:
Problems:

Description	ICD Ver.	ICD Dx Code	Snomed	Status	Diagnosed
Other muscle spasm	10	M62.838		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of cervical region	10	M99.01		active	Feb. 24, 2025, 2 p.m.
Segmental and somatic dysfunction of thoracic region	10	M99.02		active	Feb. 24, 2025, 2 p.m.
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Sprain of unspecified site of right knee, initial encounter	10	S83.91XA	11760661000119101	active	Feb. 24, 2025, 2 p.m.

Plan:
Discharge Summary: Kyara Veron
Discharge date: 03/28/2025
Functional Goals Achieved: list of conditions provided on initial visit
Functional Goals Not Achieved: all functional goals met.



Q Factor Chiropractic

Patient: Kyara Veron

DOB: 02/23/2004

Sex: F

Provider: Dr. Edward Amadi, DC

Visit: 03/28/2025 3:30PM

Chart: VEKY000001

Office: Q Factor Chiropractic

Address: 100 E. Royal Lane Suite 125, Irving, TX,
75039

Final Status: Active Tx Discharged - return as needed.

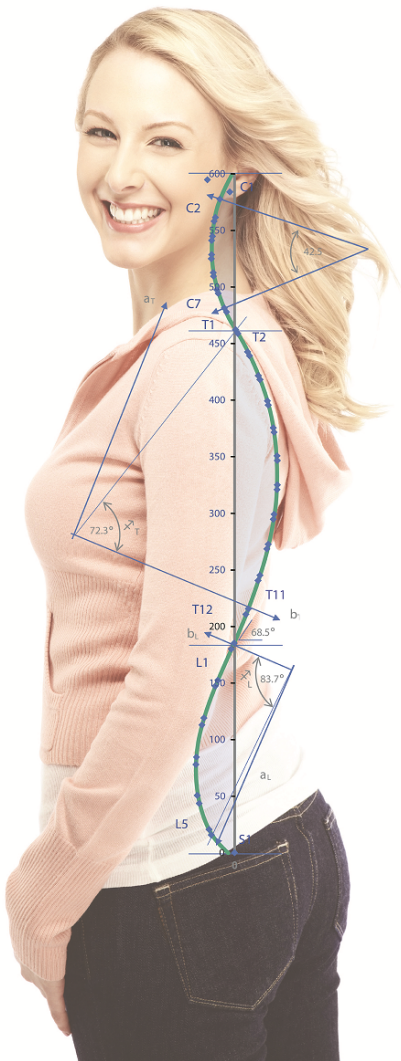
Today's Treatment

- Primary Treatment: Diversified- Chiropractic Manipulative Therapy (CMT) to the following spinal level(s): C3, C4, C5, C6 T3, T5, T4, T6, T7 L3, L4, L5
- Tx Effect: Treatment rendered, Examination performed without incident and responding as expected.
- Next Visit: MMI with residuals



PostureRay[®]
Exacting X-Ray Analysis

X-RAY Report of Findings



Prepared for : Kyara Veron

Evaluation Date : 2/24/2025

Date X-Ray Taken: 2/24/2025

Prepared by:
Q Factor Chiropractic
100 E. Royal Lane Suite 125
Irving, Texas
75039

Q Factor Chiropractic

100 E. Royal Lane Suite 125, Irving, Texas 75039 214-920-9111

X-RAY Report of Findings

Introduction

Welcome

You have chosen to be a patient at an office that utilizes posture and x-ray to evaluate your spinal alignment. While a postural analysis can provide a knowledge of gross postural/spinal abnormalities (your outside alignment), only a radiographic evaluation can provide the details of your spine's alignment and condition (your inside alignment). Your spinal alignment, any possible spinal arthritis, and disc disease (S.A.D.D.) are both conditions of interest to your doctor. With the knowledge from analyzing your spinal x-rays, your health care provider can determine a beginning clinical impression (diagnosis from any abnormalities found on your x-rays) and determine an initial program of corrective care.

What is Normal for the Spine?

Your doctor performs several levels of analyses on your spinal x-rays. First, an overall evaluation of your alignment in front-to-back radiographic views and your side radiographic views is performed. In the Front view, your spine should be straight or vertically aligned with gravity. In the Side view, your spine should have four natural curves. These four curves should be a convex forward curve in the neck (termed lordosis), a concave curve in the rib cage area (termed thoracic kyphosis), another convex forward curve in the low back (termed lumbar lordosis), and a concave curve in your sacrum-tailbone area. Figure 1 illustrates this alignment.

Figure 1.

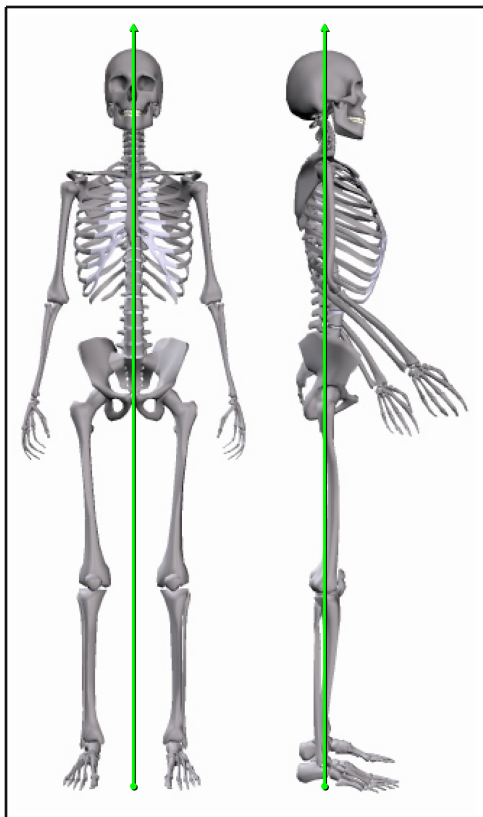


Figure 1.

Normal spinal alignment is depicted in both the front and Side views. In the front view, the center of mass of the skull, thorax, and pelvis are in a vertical line which falls between mid-stance. The spinal column is vertically aligned with respect to gravity. In the side view, the center of mass of the skull, thorax, and pelvis are in vertical alignment over the ankle. The cervical spine is lordotic, the thoracic spine is kyphotic, and the lumbar spine is lordotic.

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X-RAY Report of Findings

Introduction

For a second evaluation, your doctor looks for any obvious spinal ligament damage by observing individual spinal vertebra for any left or right misalignments in the front view and any forward or backward misalignments in the side view. Figure 2 illustrates cases of spinal ligament damage.

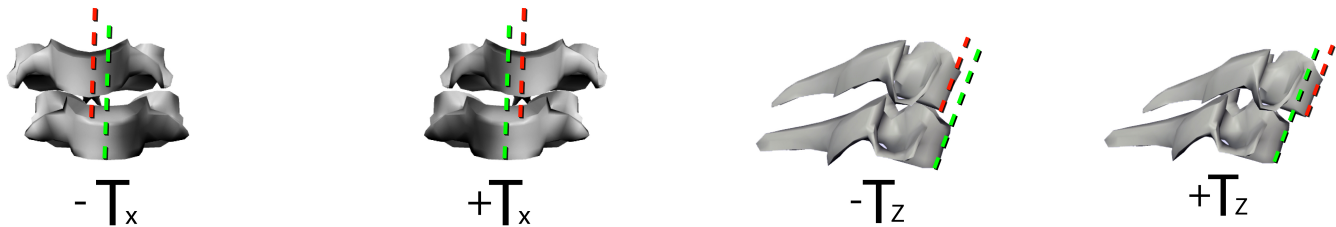


Figure 2. Ligament damage is present when a spinal vertebra does not align properly with either the vertebra immediately above it or immediately below it. In the 1st and 2nd picture, abnormal alignment of a vertebra translating left and right, signifying spinal ligament damage, is illustrated for the front view. In the 3rd picture, in the side view, backward slippage of the top vertebra is depicted. In the 4th picture, in the side view, forward slippage is shown.

For a third evaluation of your spinal x-rays, your doctor checks each vertebra for normal contour and density. This evaluation determines the state of any possible spinal arthritis and disc disease (S.A.D.D.) that you may have. Figure 3 provides an example of this analysis.



For a fourth evaluation of your spinal x-rays, your doctor checks the spacing between each pair of vertebrae. This spacing is where the spinal discs lie. Any narrowing of the normal spacing indicates disc injury and disc disease. Figure 4 presents an example of disc narrowing and disease.



Figure 4. Between the top and middle vertebrae, a normal disc spacing is seen. However, between the middle and lower vertebrae, the disc space is narrowed. This indicates that the disc has been injured and is losing its water content. While disc disease can have several causes, generally, it is a result of abnormal stress (pressures) applied to the disc from abnormal spinal alignment.

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X-RAY Report of Findings

Introduction

For a fifth evaluation of your spinal x-rays, your doctor determines the alignment of each spinal region (neck, rib cage, and low back) compared to the region immediately below by comparing each region to a vertical line in both the front view and side view. The following vertebrae should be vertically aligned with each other: C1 (first neck vertebra), T1 (first rib cage vertebra), T12 (last rib cage vertebra at the level of your kidneys), and S1 (first vertebra in your sacrum). Figure 5 illustrates this alignment for the three separate spinal regions, neck, rib cage, and low back.

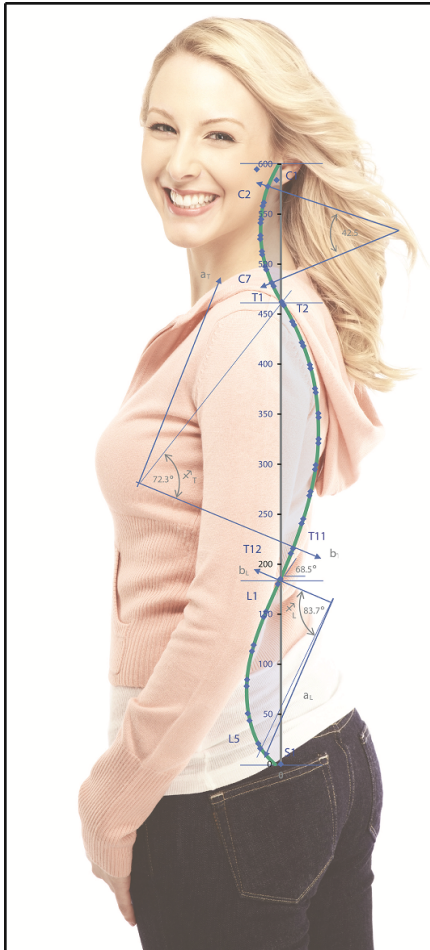


Figure 5.

Normal spinal balance from the side is when a vertical line will pass through C1, T1, T12, and S1. This can be observed all at once on a full-spine side view x-ray or for individual regions on smaller x-ray views, termed sectional x-rays. The figure to the left shows only the posterior points of each vertebra. If we look at the side view cervical (neck), C1 is aligned with T1 (thick vertical black line), with a forward convex curve termed cervical lordosis. If we just look at the side view of rib cage (thoracic), T1 is aligned with T12 and there is the presence of a concave curvature (termed thoracic kyphosis). If we look at the side view lumbar (low back), T12 is aligned with S1, with a forward convex curve termed lumbar lordosis.

For a sixth evaluation, your doctor measures any displacements of the individual spinal vertebra and/or spinal regions. These measurements are in degrees for any angular or turning (rotational) displacements and in millimeters for any sliding or shifting (translational) displacements. Figures 6 and 7 illustrate these measurements.

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X-RAY Report of Findings

Introduction

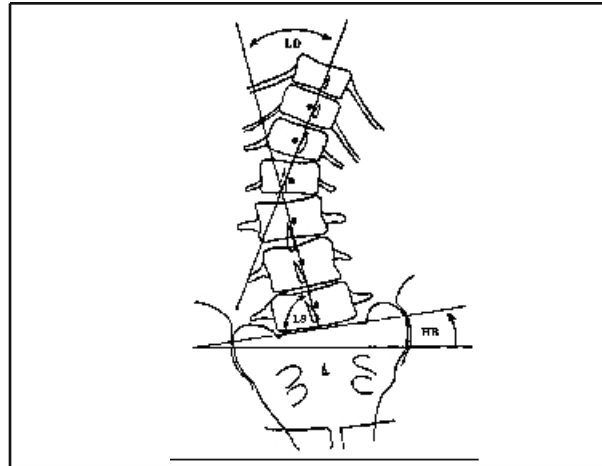
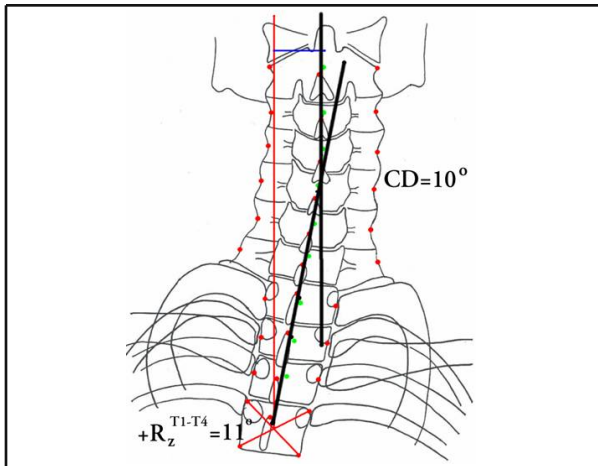


Figure 6. In the front x-ray views, lines are drawn through the centers of mass of each spinal vertebra to measure your abnormal spinal alignment. In A, an example of an analysis of abnormal spinal alignment of the neck in the front view is provided, and in B, an example of an analysis of abnormal spinal alignment of the low back is shown.

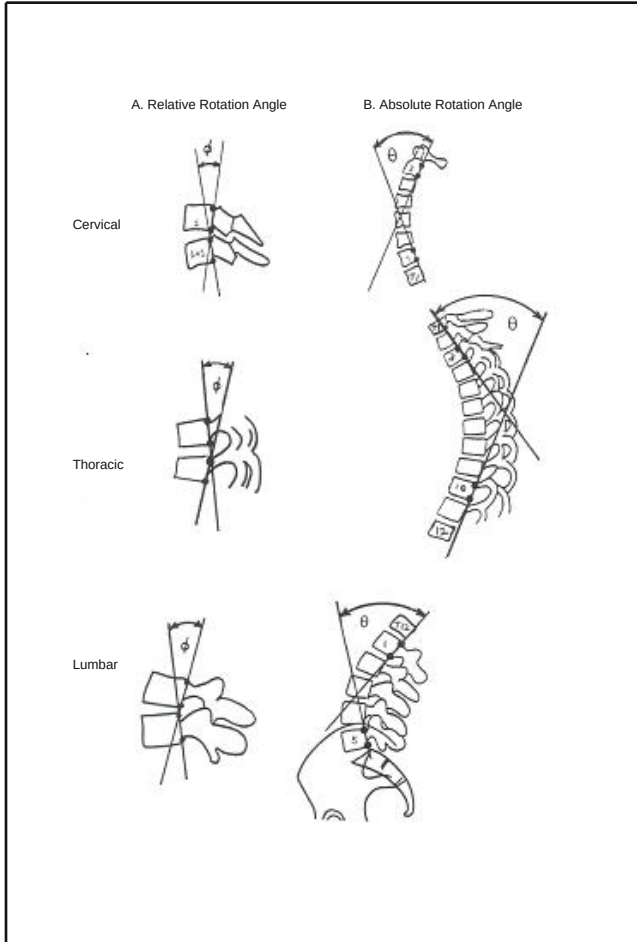


Figure 7.

In the side view, lines are drawn on the back part of each spinal vertebra. These lines are termed "Posterior Tangents". When measuring angles between adjacent posterior tangents, the angles are termed Relative Rotation Angles (RRA). When angles are formed by posterior tangents on the top and bottom vertebrae in any spinal region, these angles are termed Absolute Rotation Angles (ARA). There are precise normal values published in the scientific literature for each spinal RRA and each spinal ARA. Your alignment will be compared to these published normal values.

VERON, KYARA A DOB: 02/25/2004 (21 yo F) Acc No. 109596 DOS: 04/22/2025



Patient: VERON, KYARA A
DOB: 02/25/2004 Age: 21 Y Sex: Female

Provider: LAURA DELIN, MD
Date: 04/22/2025

Reason for Appointment

1. Low Back Pain
2. Neck Pain
3. Right Shoulder Pain

History of Present Illness

Procedure Follow-up:

Patient presents for post-procedure evaluation following Lumbar Facet Joint Injection. Patient reports significant (>80%) improvement in low back pain and function following procedure. Notes right sided low back pain seems to be more bothersome now. Associated symptoms improved following procedure including less pain with low back rotation and extension. The following activities were less painful after treatment prolonged standing and prolonged sitting. Duration of improvement: Area treated continues to improve. Patient reports persistent or recurring pain in the low back. Patient additionally reports that therapy has been helpful and that their medications are helpful. Pain complaints continue to be aggravated with lifting and bending.

RELATED INFORMATION:

Therapy: Chiro/PT (Completed: >4 weeks of therapy).

Work Status/Occupation: Regular duty.

Handedness: Right.

Affected ADLs: Physical Function, Quality of Life.

PAIN LEVELS:

Cervical Pain - Mild to Moderate.

Lumbar Pain - **Moderate to Severe.**

Right Shoulder Pain - Mild to Moderate.

Current Medications

Taking

- methIMazole
 - Methocarbamol 750 MG Tablet 1 tablet as needed for muscle spasms Orally at bedtime
 - Lidocaine 5 % Patch 1 patch remove after 12 hours Externally Once a day , Notes to Pharmacist: May substitute 4% or ointment
- Medication List reviewed and reconciled with the patient

Past Medical History

Asthma.

Hyperthyroidism.

Provider: LAURA DELIN, MD

Date: 04/22/2025

Surgical History

Denies Past Surgical History

Family History

Father: asthma

Mother: asthma

Non-Contributory

Social History

Social Drinker, Smoker.

Allergies

N.K.D.A.

Hospitalization/Major Diagnostic Procedure

Denies Past Hospitalization

Review of Systems

GENERAL:

Fever **Denies**. Unexplained Weight Loss **Denies**.

HEENT:

Vision Changes **Denies**. Headache **Denies**. Tinnitus **Denies**.

RESPIRATORY:

Constant Cough **Denies**. Shortness of breath **Denies**.

Vital Signs

HR: **115** /min, BP: **120/82** mm Hg, Wt: **164.6** lbs, BMI: **27.39** Index, Ht: 65 in, Ht-cm: 165.1 cm, Wt-kg: 74.66 kg.

Examination

GENERAL EXAMINATION:

GENERAL APPEARANCE: Well developed, well nourished, cooperative, and appears to be in no acute distress.

PSYCH: Alert and oriented to person, place, and time. The patient was able to demonstrate good judgment and reason, without abnormal affect or abnormal behaviors during the examination.

PHYSICAL EXAMINATION:

Inspection: No evidence of any skin lesions or lacerations, Antalgic gait.

Radicular/Referred Pain Pattern: Neck to right shoulder.

Sensory: Sensation is intact.

Reflexes: Not tested.

Strength: Bilateral upper and lower extremity strength normal - 5/5.

Cervical: Cervical ROM is improving, mild to moderate residual paraspinal tenderness.

Lumbar: Mild to moderate tenderness to palpation throughout the lumbar spine, ROM improved, Bilateral, R>>L, L4/5 and L5/S1 Facet pain with deep palpation. Increased with extension and rotation of the spine. (+) Facet Loading Test.

Right Shoulder: Exam deferred.

Assessments

1. Traumatic spondylopathy, lumbar region - M48.36 (Primary)
2. Traumatic spondylopathy, cervical region - M48.32
3. Pain in right shoulder - M25.511

4. Other cervical disc displacement, mid-cervical region, unspecified level - M50.220
5. Traumatic rupture of lumbar intervertebral disc, initial encounter - S33.0XXA
6. Acute pain due to trauma - G89.11
7. Decreased activities of daily living (ADL) - R68.89

PLAN: The patient continues to suffer daily pain from multiple pain generators. Recommend continued conservative care for all areas of pain in addition to the treatment plan detailed below. All patient questions were invited and answered.

DIAGNOSTICS AND RECORDS REVIEWED:

-Imaging/reports were reviewed and discussed with the patient:

MRI CERVICAL SPINE W/O CONTRAST - 03/13/2025

1. Minimal disc bulges from C3 through C6. These do not cause any significant spinal stenosis or neuroforaminal narrowing. Otherwise normal MRI of the cervical spine.

MRI LUMBAR SPINE W/O CONTRAST - 03/13/2025

1. There is a posterior paracentral to the left disc herniation at L4-5 which results in mild spinal canal stenosis and narrowing of the left lateral recess where there is probably impingement upon the passing left L5 nerve root.

2. There is also a posterior central disc protrusion at L5-S1 without significant spinal canal stenosis.

3. There is a minimal compression deformity along the superior endplate of the T12 vertebral body which does not appear to be acute.

MEDICATION: Recommend continued use of analgesics.

*Medications prescribed as below.

TREATMENT RECOMMENDATION: If residual lumbar facet pain becomes debilitating, thus limiting ADLs, will recommend an additional procedure to further cool down inflammation and facilitate healing of injuries. The goals are to restore normal function, relieve pain and symptoms, and reduce medication dependency.

1. Recommendation for treatment if symptoms persist or condition worsens: Lumbar Facet Joint Injection @ Right L4-C5 and L5-S1 Facets.

2. Recommendation for treatment if symptoms persist or condition worsens: Repeat Lumbar Facet Joint Injection @ Left L4-L5 and L5-S1 Facets.

I, BR, transcribed sections of this progress note.

Preventive Medicine

CHIRO/PT AND HOME EXERCISE PROGRAM: The importance of a doctor-directed pain management therapy program including education, manual therapies, passive modalities, activity modification, exercise, and home exercise plan (HEP) training was discussed with the patient. The benefits of a home-based exercise program for treatment of pain concurrently with pain medications was discussed with patient. Recommend to do daily to maintain/improve function. Exercises explained and given. Discussed supplemental conservative care measures including heat/ice, massage, proper diet/nutrition, weight management, support braces if indicated and activity modification.

DIAGNOSIS COUNSELING: Etiology of current pain was discussed with patient. Treatment plan and alternative options were thoroughly reviewed.

MEDICATION COUNSELING: Risks, benefits, and alternatives of medications were thoroughly discussed with patient.

VERON, KYARA A DOB: 02/25/2004 (21 yo F) Acc No. 109596 DOS: 04/22/2025

Follow Up

2 weeks for re-evaluation.

A handwritten signature in cursive script that reads "Laura Delin". The ink is light gray and the signature is centered horizontally.

Electronically signed by LAURA DELIN on 04/23/2025 at 09:26 AM CDT

Sign off status: Completed

Provider: LAURA DELIN, MD

Date: 04/22/2025

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VERON, KYARA A DOB: 02/25/2004 (21 yo F) Acc No. 109596 DOS: 05/06/2025



Patient: VERON, KYARA A
DOB: 02/25/2004 Age: 21 Y Sex: Female

Provider: LAURA DELIN, MD
Date: 05/06/2025

Reason for Appointment

1. Neck Pain
2. Low Back Pain
3. Right Shoulder Pain

History of Present Illness

Follow-up:

Patient is following up from conservative care for re-evaluation. Patient reports persistent pain in the low back. Pain complaints are accompanied by symptoms of burning sensations. Pain complaints continue to be aggravated by lifting, work duties, and bending. Patient reports improved pain in the neck and right shoulder. Patient additionally reports that their medications are helpful.

RELATED INFORMATION:

Therapy: Chiro/PT (Completed: >4 weeks of therapy).
Work Status/Occupation: Regular duty.
Handedness: Right.
Affected ADLs: Physical Function, Quality of Life.

PAIN LEVELS:

Cervical Pain - Mild to Moderate.
Lumbar Pain - **Moderate to Severe**.
Right Shoulder Pain - Mild to Moderate.

Current Medications

Taking

- methIMAzole
 - Methocarbamol 750 MG Tablet 1 tablet as needed for muscle spasms Orally at bedtime
 - Lidocaine 5 % Patch 1 patch remove after 12 hours Externally Once a day , Notes to Pharmacist: May substitute 4% or ointment
- Medication List reviewed and reconciled with the patient

Past Medical History

Asthma.
Hyperthyroidism.

Surgical History

Denies Past Surgical History

Provider: LAURA DELIN, MD

Date: 05/06/2025

Family History

Father: asthma
Mother: asthma
Non-Contributory

Social History

Social Drinker, Smoker.

Allergies

N.K.D.A.

Hospitalization/Major Diagnostic Procedure

Denies Past Hospitalization

Review of Systems

GENERAL:

Fever **Denies**. Unexplained Weight Loss **Denies**.

HEENT:

Vision Changes **Denies**. Headache **Denies**. Tinnitus **Denies**.

RESPIRATORY:

Constant Cough **Denies**. Shortness of breath **Denies**.

GASTROINTESTINAL:

Hematochezia **Denies**. Constipation **Denies**. Hematemesis **Denies**.

GENITOURINARY:

Urinary Incontinence **Denies**.

SKIN:

Rashes **Denies**. Tumors **Denies**.

NEUROLOGIC:

Speech Problems **Denies**.

PSYCHIATRIC:

Depression **Denies**. Anxiety **Denies**.

CARDIOVASCULAR:

Pedal Edema **Denies**. Palpitations **Denies**.

Vital Signs

HR: **122** /min, BP: **122/92** mm Hg, Wt: **163.4** lbs, BMI: **27.19** Index, Ht: 65 in, Ht-cm: 165.1 cm, Wt-kg: 74.12 kg.

Examination

GENERAL EXAMINATION:

GENERAL APPEARANCE: Well developed, well nourished, cooperative, and appears to be in no acute distress.

PSYCH: Alert and oriented to person, place, and time. The patient was able to demonstrate good judgment and reason, without abnormal affect or abnormal behaviors during the examination.

PHYSICAL EXAMINATION:

Inspection: No evidence of any skin lesions or lacerations, Antalgic gait.

Radicular/Referred Pain Pattern: Neck to right shoulder.

Sensory: Sensation is intact.

Reflexes: Not tested.

Strength: Bilateral upper and lower extremity strength normal - 5/5.

Cervical: Cervical ROM is improving, mild to moderate residual paraspinal tenderness.

VERON, KYARA A DOB: 02/25/2004 (21 yo F) Acc No. 109596 DOS: 05/06/2025

Lumbar: Mild to moderate tenderness to palpation throughout the lumbar spine, ROM improved, Bilateral, R>>L, L4/5 and L5/S1 Facet pain with deep palpation. Increased with extension and rotation of the spine. (+) Facet Loading Test.

Right Shoulder: Exam deferred.

Assessments

1. Traumatic spondylopathy, lumbar region - M48.36 (Primary)
2. Traumatic spondylopathy, cervical region - M48.32
3. Pain in right shoulder - M25.511
4. Other cervical disc displacement, mid-cervical region, unspecified level - M50.220
5. Traumatic rupture of lumbar intervertebral disc, initial encounter - S33.0XXA
6. Acute pain due to trauma - G89.11
7. Decreased activities of daily living (ADL) - R68.89
8. Fear of Needles - F40.231
9. Low Pain Threshold/Severe Pain - G97.81
10. Pre-procedural Anxiety - F44.9

PLAN: The patient continues to suffer daily pain from multiple pain generators. Recommend continued conservative care for all areas of pain in addition to the treatment plan detailed below. All patient questions were invited and answered.

DIAGNOSTICS AND RECORDS REVIEWED:

-Imaging/reports were reviewed and discussed with the patient:

MRI CERVICAL SPINE W/O CONTRAST - 03/13/2025

1. Minimal disc bulges from C3 through C6. These do not cause any significant spinal stenosis or neuroforaminal narrowing. Otherwise normal MRI of the cervical spine.

MRI LUMBAR SPINE W/O CONTRAST - 03/13/2025

1. There is a posterior paracentral to the left disc herniation at L4-5 which results in mild spinal canal stenosis and narrowing of the left lateral recess where there is probably impingement upon the passing left L5 nerve root.

2. There is also a posterior central disc protrusion at L5-S1 without significant spinal canal stenosis.

3. There is a minimal compression deformity along the superior endplate of the T12 vertebral body which does not appear to be acute.

MEDICATION: Recommend continued use of analgesics.

INTERVENTIONAL TREATMENT: Facetogenic lumbar pain is a direct result of the trauma and impact of the accident causing irritation and inflammation of the facet joints confirmed by history and physical exam, which demonstrates exacerbation of pain with extension and rotation of the lumbar spine. Pain and symptoms have been refractory to conservative therapy and analgesics. Facetogenic lumbar pain is inhibiting improvement of ADLs and requiring frequent use of analgesics. Will plan for one round of facet injections to treat painful and inflamed facet joints in the lumbar spine. The goals are to restore normal function, relieve pain and symptoms, and reduce medication dependency. Medical records and imaging were thoroughly reviewed with the patient. Risks, benefits, and alternatives to the procedure were discussed, and all patient questions were answered. Patient requests MAC anesthesia be discussed with anesthesia professional due to low pain threshold, anxiety and fear regarding the procedure.

I, MK, transcribed sections of this progress note.

Treatment

Provider: LAURA DELIN, MD

Date: 05/06/2025

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VERON, KYARA A DOB: 02/25/2004 (21 yo F) Acc No. 109596 DOS: 05/06/2025

1. Traumatic spondylopathy, lumbar region

Procedure: LUMBAR FACET JOINT INJECTION RIGHT L4-L5 AND L5-S1 (64493, 64494).

MAC

Preventive Medicine

CHIRO/PT AND HOME EXERCISE PROGRAM: The importance of a doctor-directed pain management therapy program including education, manual therapies, passive modalities, activity modification, exercise, and home exercise plan (HEP) training was discussed with the patient. The benefits of a home-based exercise program for treatment of pain concurrently with pain medications was discussed with patient. Recommend to do daily to maintain/improve function. Exercises explained and given. Discussed supplemental conservative care measures including heat/ice, massage, proper diet/nutrition, weight management, support braces if indicated and activity modification.

DIAGNOSIS COUNSELING: Etiology of current pain was discussed with patient. Treatment plan and alternative options were thoroughly reviewed.

MEDICATION COUNSELING: Risks, benefits, and alternatives of medications were thoroughly discussed with patient.

SMOKING/VAPING CESSATION COUNSELING: Patient counseled on the benefits of quitting smoking/vaping for a healthy lifestyle.

Follow Up

2 weeks after procedure for re-evaluation



Electronically signed by LAURA DELIN on 05/07/2025 at 08:23 AM CDT

Sign off status: Completed

Provider: LAURA DELIN, MD

Date: 05/06/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

Generated for Printing/Faxing/eTransmitting on: 07/27/2025 10:57 PM CDT

VERON, KYARA A DOB: 02/25/2004 (21 yo F) Acc No. 109596 DOS: 05/27/2025



VERON, KYARA A

21 Y old Female, DOB: 02/25/2004

2824 LAKE GARDENS DR, IRVING, TX-75060

Home: 214-529-7594

Surgeon: TORRENCE J STEPTEAU, M.D.

05/27/2025

TORRENCE J STEPTEAU, M.D.

OPERATIVE REPORT

Pre-op. Diagnosis:

- 1.TRAUMATIC SPONDYLOPATHY LUMBAR M.48.36

Post-op. Diagnosis:

- 1.Same as pre-op diagnosis

Anesthesia:

Indications:

Details of Procedure:

TITLE OF PROCEDURE: Lumbar Facet Joint Injection at the RIGHT L4-L5 & L5-S1 Facets Under Fluoroscopic Guidance.

1. Visualization of lumbar facet joints and sacroiliac joints.
2. Placement of needles in the FACET JOINTS at level: RIGHT L4-L5 AND RIGHT L5-S1; TWO DIFFERENT PUNCTURE SITES.
3. Injection of radiopaque dye in the lumbar facet joint space X 2.
4. Performance of lumbar facetogram X 2.
5. Interpretation of lumbar facetogram X 2.
6. Therapeutic and/or Diagnostic Injection of DEXAMETHASONE and bupivacaine into the lumbar facet joint space X 2.

ANESTHESIA: M.A.C.

ESTIMATED BLOOD LOSS: None

PROCEDURE:

Written informed consent discussing the risks, benefits, and alternative therapies was obtained prior to the start of the procedure. The patient was brought to the operating room and placed in the prone position. The lumbar spine was prepped and draped in the usual sterile fashion with chlorhexidine solution.

Under FLUOROSCOPIC GUIDANCE, the facet joints were identified at the RIGHT L4-L5 AND RIGHT L5-S1 facets (2 LOCATIONS). Local anesthetic was provided to the overlying skin and soft tissues utilizing 1% lidocaine (5mL).

TWO 22-gauge spinal needles were placed onto the facet joints via a dorsal approach at the RIGHT L4-L5 AND RIGHT L5-S1 facets (2 LOCATIONS). The injection of 0.25ml of Omnipaque-240 was performed at each FACET LOCATION x2. Instilled contrast outlines the facet joint spaces, confirming the location of needle tips. Images of the facetograms were obtained in

Progress Note: TORRENCE J STEPTEAU, M.D. 05/27/2025

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VERON, KYARA A DOB: 02/25/2004 (21 yo F) Acc No. 109596 DOS: 05/27/2025

multiple projections. The interpretation of the LUMBAR FACETOGRAMS (2 FACETOGRAMS) demonstrates the needle tips to be in the correct position, with spread of contrast to the targeted areas. A HARD COPY is available upon request.

INJECTION OF DEXAMETHASONE INTO THE FACET JOINT SPACE X 2: Following the verification of needle positions, a mixture of 5mg of Particulate and Preservative Free DEXAMETHASONE (LOT# 6036718 EXP: 12/2026) and 0.5mL of 0.25% MARCAINE was injected at each FACET JOINT LOCATION X 2.

The patient tolerated the procedure without complications and was transferred to the recovery area. The patient was monitored for an appropriate length of time after the injection and then discharged to home in good condition with a driver. The patient was instructed to refrain from driving for 12 hours and to contact the office immediately if any of the following were noted: new or worsening pain in the back, loss of bladder or bowel control, rash or hives, numbness or weakness of the legs, redness or drainage from needle insertion site, or any other concerning symptom.

FLUOROSCOPY TIME: Less than 1.0 minutes.

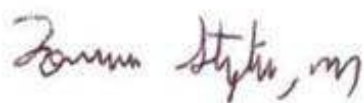
The patient was instructed to continue conservative therapy post-procedurally.

Specimens:

Complications:

Findings:

Notes:



Electronically signed by TORRENCE STEPTEAU on 05/27/2025 at 04:07 PM CDT

Sign off status: Completed

Arlington Momentum
950 NORTH DAVIS DRIVE
ARLINGTON, TX 76012-3247
Tel: 214-557-4111
Fax: 972-972-8660

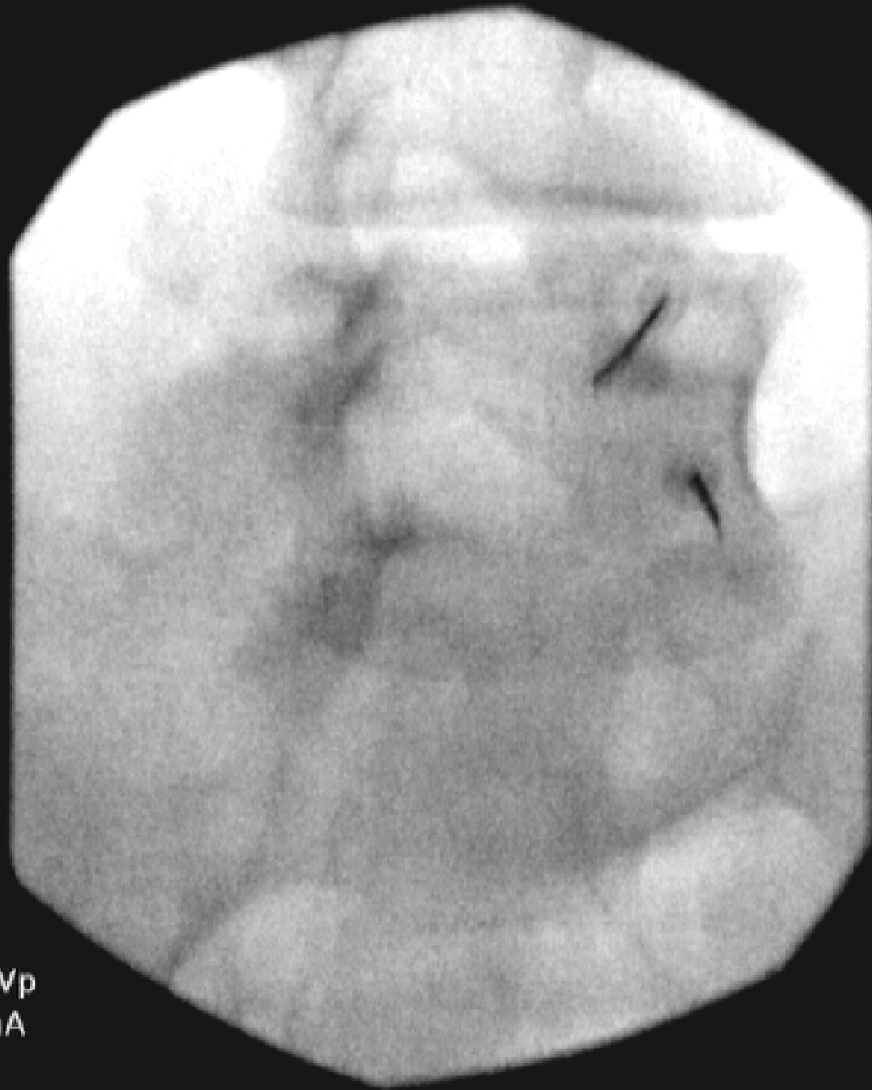
Progress Note: TORRENCE J STEPTEAU, M.D. 05/27/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

VERON
KYARA
2/25/2004

05/27/2025
10:28:19 AM

51-☀-
68-☾-



100 kVp
1.52 mA

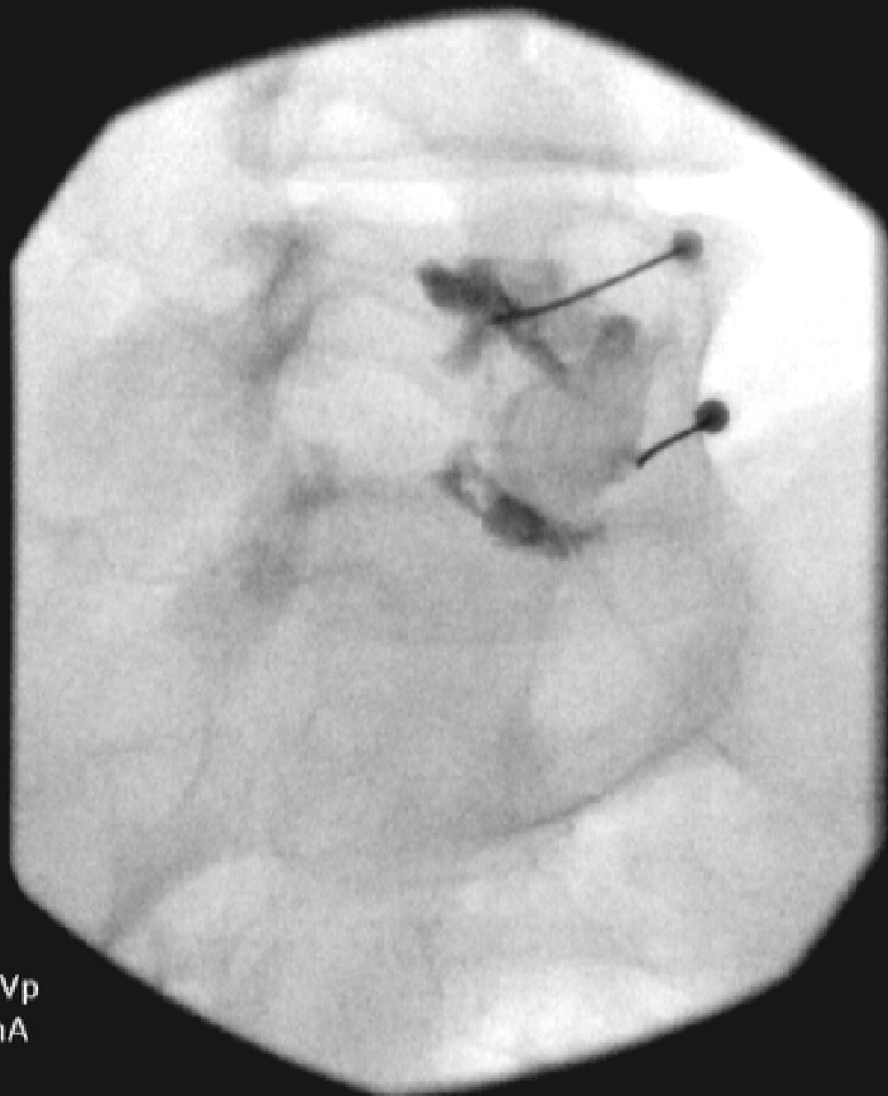
1

OEC

VERON
KYARA
2/25/2004

05/27/2025
10:28:39 AM

56 
53 



100 kVp
1.52 mA

2

OEC

Anesthesia Pre-Op Assessment and Intraoperative Record

PATIENT IDENTIFICATION:

VERON, KYARA

02/25/2004

05/27/2025

10:00 AM

Date: 05/27/2025

H 65 W 163		ASA 1		Diagnosis: TRAUMATIC SPONDYLOPATHY, LUMBAR REGION	
Medications: METHIMAZOLE, METHOCARBAMOL, LIDOCAINE PATCH		Allergies: None		Procedure: RIGHT L4-L5 & L5-S1 LUMBAR FACET JOINT INJECTION	
Cardiovascular: HTN, MI, Angina, CAD, CHF, Arrhythmia, DVT/PE, Hypertlipidemia		Neurological: TIA, Anxiety, CVA, Depression, Seizures, Denies, ETOH/Drug Abuse		Pulmonary: OSA, COPD, SOB, Asthma, Smoker, Denies	
PRE-OP ASSESSMENT: Chart Reviewed, Permit Signed		MONITORS AND EQUIPMENT: Steth, Precord, Esoph, Non-Invasive B/P, Pulse Oximeter, End Tidal CO₂		AIRWAY MANAGEMENT: Intubation: ETT, LMA, Tube Size, Attempts, Blade, Secured at, Grade, view, Airway: Oral, Nasal, Airway Size	
PATIENT SAFETY: Anes. Machine/Monitors Checked, Pressure points padded, Axillary Roll, Eye Care		PRE-INDUCTION EVALUATION: Status: Changed, Unchanged		REMARKS: EXCITMENT & SEDATION	
TIME: 1030		START: 1030		STOP: 1040	
O₂ (L/min): 3		Anesthesia: 1030/1040		Procedure: 1039/1037	
Versed (mg): 40		Time Out: 1032		REMARKS: EXCITMENT & SEDATION	
Fentanyl (mcg): 100/100 (200)		Antibiotics: at		Surgeon declined	
Lidocaine (mg): 51		Recovery: 1046		Time: 1046	
Propofol (mg): 97		Recovery: 1046		Time: 1046	
EKG: 51		Recovery: 1046		Time: 1046	
FIO₂: 97		Recovery: 1046		Time: 1046	
SaO₂%: 97		Recovery: 1046		Time: 1046	
ETCO₂ mm Hg: 220		Recovery: 1046		Time: 1046	
POSITION: Supine		Recovery: 1046		Time: 1046	
Tidal Volume: -SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV		Recovery: 1046		Time: 1046	
Peak Inspiratory Pressure: -SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV		Recovery: 1046		Time: 1046	
Respiratory Rate: -SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV		Recovery: 1046		Time: 1046	
IVF: -SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV		Recovery: 1046		Time: 1046	
EBL: -SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV		Recovery: 1046		Time: 1046	
UO: -SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV, SV		Recovery: 1046		Time: 1046	
POST-ANESTHESIA DISCHARGE SUMMARY: Meets requirements for discharge from anesthesia, Discharge instructions given to patient by RN and/or surgeon		Cardiopulmonary Status: Stable, Awake, Adequate, No, None		ANESTHESIA PROVIDER: CRNA Signature: R. Ramey, Printed Name: RAMEY	
Date/Time: 5/27/25		Signature: R. Ramey		CRNA Signature: R. Ramey, Printed Name: RAMEY	



PROCEDURE COST ESTIMATE ONLY

PATIENT NAME: VERON, KYARA
DATE OF SERVICE: 03/24/2025

DOB: 02/25/2004

PROCEDURE: 1. CERVICAL FACET INJECTION TWO LEVELS AT RIGHT C4-C5 AND C5-C6

CODE	DESCRIPTION	QTY	AMOUNT
64490	Cervical Facet Steroid Injection (1st Level)	1	\$4,743.75
64491	Cervical Facet Steroid Injection (2nd Level)	1	\$2,429.50
01992	Anesthesia	1	1,800.00

TOTAL AMOUNT OF INVOICE \$ **8,973.25**

9441 Lyndon B. Johnson Freeway, Dallas, TX 75243
Office: (214) 557-4111 Fax: (214) 764-7611

This is an estimate of charges based on the recommendation provided by above mentioned physician. Should it become necessary for the provider to perform any additional procedures other than those listed, the charge amount is subject to change.

This estimate is valid for 180 days from the date of the estimate. This price is an approximate charge for this procedure. For any questions, please contact our office at (214) 557-4111.

BLUE STONE JV LLP
PO BOX 746006
Atlanta, GA 30374-6006
Phone: (833)269-8017
Fax: (972)497-4104
Tax ID:

Date Created: Sunday, July 20, 2025 7:15 PM

Patient: 3052495 - VERON, KYARA

DATE	REF. DOCTOR	DOCTOR	PROCEDURE	MODIFIER	ICD	CHARGES	PAYMENTS	ADJUSTMENTS	BALANCE	PAYSOURCE
03/13/2025	AMADI, EDWARD	DEUELL, BRIAN	72141 - MRI CERVICAL SPINE W/O		M50.21, M50.221, M50.222	\$2,119.00				
						\$2,119.00	\$0.00	\$0.00	\$2,119.00	
03/13/2025	AMADI, EDWARD	BURGARD T, JOHN	72148 - MRI LUMBAR SPINE W/O		M48.061, M51.26, M51.27	\$2,119.00				
						\$2,119.00	\$0.00	\$0.00	\$2,119.00	
03/13/2025	AMADI, EDWARD	DEUELL, BRIAN	73721 - MRI JOINT LOWER EX	RT	M25.551	\$2,119.00				
						\$2,119.00	\$0.00	\$0.00	\$2,119.00	
Account Total:						\$6,357.00	\$0.00	\$0.00	\$6,357.00	



Touchstone Imaging Las Colinas
 440 W. I-635, Suite 110, Plaza II
 Irving, TX 75063
 Phone #: (214)647-6161
 Fax: (214)647-6162

Name: KYARA VERON
 Patient ID: 9913076143
 Age: 21Y 4M
 DOB: 2/25/2004
 Acc #: 18667914

Exam Date: 3/13/2025 03:12 PM
 Exam Name: MRI Cervical Spine Without Contrast |
 72141
 Reason:: Cervicalgia | M542
 Referrer: EDWARD AMADI, DR
 2nd Referrer:

PROCEDURE: MRI Cervical Spine Without Contrast

INDICATION: Cervicalgia. Right side shoulder pain.

COMPARISON: None.

TECHNIQUE: Multiplanar and multisequence MR imaging of the cervical spine was performed without the use of intravenous contrast.

FINDINGS:

The cervical spine is anatomically aligned. No abnormal bone marrow signal is seen in the vertebral bodies. No abnormal signal is seen in the spinal cord.

The visualized structures of the posterior fossa image normally. The soft tissues of the neck image normally.

C2-C3: No significant abnormality.

C3-C4: Minimal bulging of the disc without spinal stenosis or neuroforaminal narrowing.

C4-C5: Minimal bulging of the disc without spinal stenosis or neuroforaminal narrowing.

C5-C6: Minimal bulging of the disc without spinal stenosis or neuroforaminal narrowing.

C6-C7: No significant abnormality.

C7-T1: No significant abnormality.

IMPRESSION:

Minimal disc bulges from C3 through C6. These do not cause any significant spinal stenosis or neuroforaminal narrowing. Otherwise normal MRI of the cervical spine.

This exam was interpreted and dictated from 76104-RDALL.

This report was dictated at 76104-RDALL and electronically signed by Brian Deuell, M.D. on 3/13/2025 3:54 PM CDT.

Dr. Brian Deuell, M.D. is affiliated with American Radiology Associates.

Report Electronically Signed by: Brian Deuell MD
Report Electronically Signed on: 3/13/2025 03:54 PM

Patient Name: KYARA VERON

Exam:

Patient ID: 9913076143
Completed Date: 3/13/2025 03:12 PM
Transcribed By: Brian Deuell MD
Transcribed Date: 3/13/2025 03:54 PM

MRI Cervical Spine Without Contrast |
72141
Acc #: 18667914
Interpreting Rad: Brian Deuell MD
Dictated Date: 3/13/2025 03:54 PM
Finalized Date: 3/13/2025 03:54 PM



Touchstone Imaging Las Colinas
 440 W. I-635, Suite 110, Plaza II
 Irving, TX 75063
 Phone #: (214)647-6161
 Fax: (214)647-6162

Name: KYARA VERON
 Patient ID: 9913076143
 Age: 21Y 4M
 DOB: 2/25/2004
 Acc #: 18667916

Exam Date: 3/13/2025 02:59 PM
 Exam Name: MRI Lumbar Spine Without Contrast |
 72148
 Reason:: Cervicalgia | M542
 Referrer: EDWARD AMADI, DR
 2nd Referrer:

PROCEDURE: MRI OF THE LUMBAR SPINE WITHOUT CONTRAST

INDICATION: Low back pain

COMPARISON: none

TECHNIQUE: Multisequence, multiplanar technique without IV contrast

FINDINGS: There is normal alignment within the lumbar spine down through the lumbosacral junction. The marrow signal throughout the lumbar spine is within normal limits. The conus medullaris has a normal appearance and terminates appropriately at the L1-2 level. The paravertebral tissues are unremarkable.

There is a minimal compression deformity along the superior endplate of the T12 vertebral body with mild endplate irregularity. This does not appear to be acute.

No abnormalities are seen at the T12-L1 or L1-2 levels.

At L2-3, there are no significant abnormalities.

At L3-4, there is mild facet arthrosis on the left. The spinal canal dimensions are ample as are the foramina.

At L4-5 there is mild to moderate disc space narrowing. There is an annular disc bulge with a moderate-sized posterior paracentral to the left disc herniation. Mild spinal canal stenosis is present. There is narrowing of the left lateral recess where there is probably impingement upon the passing left L5 nerve root. The foramina are ample bilaterally.

At L5-S1, there is an annular disc bulge. Moderate facet overgrowth is present on the left with mild to moderate facet overgrowth on the right. There is a posterior central disc protrusion. The spinal canal dimensions are within normal limits. There is mild left-sided foraminal narrowing.

IMPRESSION:

1. There is a posterior paracentral to the left disc herniation at L4-5 which results in mild spinal canal stenosis and narrowing of the left lateral recess where there is probably impingement upon the passing left L5 nerve root.
2. There is also a posterior central disc protrusion at L5-S1 without significant spinal canal stenosis.
3. There is a minimal compression deformity along the superior endplate of the T12 vertebral body which does not appear to be acute.

This report was dictated at 76262-RWBURG and electronically signed by John D. Burgardt, M.D. on 3/13/2025 3:56 PM CDT.

Dr. John Burgardt, M.D. is affiliated with American Radiology Associates.

Report Electronically Signed by: John Burgardt

Report Electronically Signed on: 3/13/2025 03:56 PM

Patient Name: KYARA VERON
Patient ID: 9913076143
Completed Date: 3/13/2025 02:59 PM
Transcribed By: John Burgardt
Transcribed Date: 3/13/2025 03:56 PM

Exam: MRI Lumbar Spine Without Contrast |
72148
Acc #: 18667916
Interpreting Rad: John Burgardt
Dictated Date: 3/13/2025 03:56 PM
Finalized Date: 3/13/2025 03:56 PM



Touchstone Imaging Las Colinas
 440 W. I-635, Suite 110, Plaza II
 Irving, TX 75063
 Phone #: (214)647-6161
 Fax: (214)647-6162

Name: KYARA VERON
 Patient ID: 9913076143
 Age: 21Y 4M
 DOB: 2/25/2004
 Acc #: 18667925

Exam Date: 3/13/2025 02:36 PM
 Exam Name: MRI Lower Ext Joint (Hip) WO Contrast
 RT | 73721
 Reason:: Cervicalgia | M542
 Referrer: EDWARD AMADI, DR
 2nd Referrer:

PROCEDURE: MRI Lower Ext Joint (Hip) WO Contrast RT

INDICATION: Right hip pain.

COMPARISON: None.

TECHNIQUE: Multiplanar and multisequence MR imaging of the right hip was performed without the use of intravenous contrast.

FINDINGS:

Bone and joint:

Alignment of the right hip is anatomic. Small marginal osteophytes around the acetabular rim and femoral head. No bone marrow signal abnormality around the right hip. The lateral center edge angle is 40 degrees, which is on the upper limits of normal. The alpha angle is normal. The hyaline cartilage is intact. Suspected small tear in the posterosuperior labrum on coronal series 7 images 12 and 13. No paralabral cyst. The labrum is otherwise normal. No significant joint effusion.

The large field-of-view imaging of the contralateral left hip, sacroiliac joints, and pubic symphysis show no significant abnormality.

Soft tissues:

The tendon origins and insertions about the hip are intact. No muscle edema or atrophy. No extra capsular fluid collection or edema. The rectus abdominis-hip adductor aponeurosis is intact. No significant narrowing of the quadratus femoris space.

The sciatic nerve is normal in signal intensity and morphology. The femoral neurovascular bundle is intact. No lymphadenopathy. In the posterior right side of the pelvis, a 3.3 cm round lesion with hyperintense T1 signal and loss of signal on T2 and proton-density fat-suppressed images is likely a hemorrhagic cyst.

IMPRESSION:

1. Question a small tear in the posterosuperior labrum on coronal images 12 and 13. Confirmation could be confirmed by MR arthrography of the right hip if necessary.
2. Right ovarian lesion in the posterior right pelvis favored to represent a hemorrhagic cyst, measuring 3.3 cm.

This exam was interpreted and dictated from 76104-RDALL.

This report was dictated at 76104-RDALL and electronically signed by Brian Deuell, M.D. on 3/13/2025 3:51 PM CDT.

Dr. Brian Deuell, M.D. is affiliated with American Radiology Associates.

Report Electronically Signed by: Brian Deuell MD
Report Electronically Signed on: 3/13/2025 03:51 PM

Patient Name: KYARA VERON
Patient ID: 9913076143
Completed Date: 3/13/2025 02:36 PM
Transcribed By: Brian Deuell MD
Transcribed Date: 3/13/2025 03:51 PM

Exam: MRI Lower Ext Joint (Hip) WO
Contrast RT | 73721
Acc #: 18667925
Interpreting Rad: Brian Deuell MD
Dictated Date: 3/13/2025 03:51 PM
Finalized Date: 3/13/2025 03:51 PM

Patient Profile Report Include Prescription Detail
ASP CARES: 4309 Lemmon Ave Dallas TX 75219 Phone: (214)331-4500 FAX: (214)331-4507

Generated: 09/05/2025 09:52:57 Patient Selection: VERON, KYARA (21)
Fill Date Range: Fri 02/21/2025 - Drug Selection: All Drugs, Including Inactive
Doctor Selection: All Doctors, Including Inactive Profiled Prescriptions: Omit
Sort By: Rx Number

RxNumber	OrigDate	DiscDate	Doctor	TtlQty	QtyLeft	Qty/Ref	Drug Written For			
	RefDate	Drug Dispensed			NDC Number	Ext	Qty	Days	Price	Insurance
Patient: VERON, KYARA			Address: 2824 LAKE GARDENS DR IRVING TX 75060							
Primary Ins: LOP		ID:	Grp:	Plan: 00000		Rel:	Per:			
Phone: (214)529-7594		Sex: F	Birthdate: 02/25/2004	Age: 21						
608473	03/26/25	VASQUEZ, DANIELLE			60	30	30 METHOCARBAMOL			
	04/07/25	METHOCARBAMOL 750MG PO TAB				70010-0770-05	30	30	249.00	LOP
608474	03/26/25	VASQUEZ, DANIELLE			60	30	30 LIDOCAINE			
	04/07/25	LIDOCAINE 5% TO PATCH				82347-0505-05	30	30	249.00	LOP
									498.00	
									498.00	

RPh Signature: _____ Date: _____